



Comhairle na nDochtúirí Leighis
Medical Council

Medical Council

Medical Workforce Intelligence Consolidated Report 2022

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Glossary

ARAF	Annual Retention Application Form
BMQ	Basic Medical Qualification
CPD	Continuing Professional Development
DOH	Department of Health
EEA	European Economic Area
EU	European Union
EWTD	European Working Time Directive
GP	General Practitioner
HSE	Health Service Executive
IMG	International Medical Graduate
MPA	Medical Practitioners Act
NCHD	Non-Consultant Hospital Doctor
NDTP	National Doctors Training and Planning
OECD	Organisations for Economic Co-operation and Development
PGTB	Post Graduate Training Body
RMP	Registered Medical Practitioner
VW	Voluntary Withdrawal
WHO	World Health Organisation
WIR	Workforce Intelligence Report

Introduction

This Workforce Intelligence Report analyses and presents data provided by all doctors who have registered with the Medical Council for the first time, retained their registration and voluntarily withdrawn from the Medical Register ('the Register') throughout 2022. Maintaining this Register is fundamental to the Medical Council's remit of protecting the public, and data held on the Register provides valid and robust insight into the medical workforce in Ireland. This report presents an analysis of quantitative and qualitative data on registration and divisional status, employment role, working hours, training status, county of practise and voluntary withdrawal from the Register.

The data contained within this report should be used to inform recruitment and retention strategies, medical education and training development and health policy in Ireland. This robust data, combined with employment data, can support effective planning for a strong and sustainable medical workforce. It is currently being used by the Department of Health (DOH) to inform the Health and Social Care Workforce Planning Strategy and Action Plan. Ensuring that the health system has the right doctors in the right place and at the right time rests on a decisive and collaborative response that supports not only our medical workforce, but all teams working in the health sector.

The data reinforce issues highlighted in previous iterations of this report and other research reports, relating to, for example, a shortage of consultants and a need for increased training opportunities for non-consultant hospital doctors (NCHDs), as well as a challenging working environment.^{1,2,3} Such conditions can affect doctors' willingness to stay and work in Ireland despite initial intentions to remain, as described in Your Training Counts 2019-2020.

The Council is keen to play an engaging role in supporting the development and implementation of a strategic workforce framework for doctors, as an integral part of a wider healthcare workforce strategy. Effective and strategic workforce planning can be achieved through concerted collaborative working amongst policymakers, educators, regulators, planners and employers, including the DOH, HSE, the Forum of Irish Postgraduate Medical Training Bodies and Medical Schools.

¹ Brugha, R., Clarke, N., Hendrick, L., & Sweeney, J. (2021). Doctor retention: a cross-sectional study of how Ireland has been losing the battle. *International journal of health policy and management*, 10(6), 299.

² Humphries, N., Creese, J., Byrne, J.-P., & Connell, J. (2021). COVID-19 and doctor emigration: the case of Ireland. *Human resources for health*, 19(1), 1-10.

³ McLoughlin, C., Casey, S., Feeney, A., Weir, D., Abdalla, A. A., & Barrett, E. (2021). Burnout, work satisfaction, and well-being among non-consultant psychiatrists in Ireland. *Academic Psychiatry*, 45, 322-328.

WORKFORCE INTELLIGENCE REPORT 2022 IN NUMBERS

Doctors who registered with the Medical Council for the first time in 2022

3,008
Doctors



1,581 males (52.6%)

1,427 females (47.4%)



**867 Irish BMQ
(28.8%)**



**2,141 International BMQ
(71.2%)**

Doctors who retained their place on the Medical Council's Register in 2022

23,108
Doctors
retained



**22,317
(96.6%)
Clinically
active**



**18,839
(81.5%)
Practicing
in Ireland**

**10,017
(53.2%)
Male
8,822
(46.8%)
Female**

**Mean age: 43.9
21.3% aged 55
and above**



**NCHD in training
3,956
NCHD not in training
3,959**

International BMQ 6,977

**Reported working
more than 48hs a
week 5,347 (28.6%)**

**Trainee Specialist Registration: 3,077
Specialist Registration: 9,740
General Registration: 5,870
Supervised Registration: 152**



Doctors who voluntarily withdrew from the Register in 2022

**1,341 Doctors
voluntarily
withdrew**



**748 males (55.8%)
593 females (44.2%)**

**Irish BMQ 586 (43.7%)
International BMQ 755 (56.3%)**

Summary of Key Insights

Analysis of registration and voluntary withdrawal data have highlighted key insights and issues that can inform strategic and integrated medical workforce planning,

1. The General Division

Doctors on the general division account for a substantial proportion of all clinically active doctors in Ireland. The general division was created as a transition for doctors progressing from internship to the trainee specialist division, and as such does not offer specialist training. Over-reliance on doctors on the general division to deliver specialist care can have potential implications for patient safety.

2. Non-Consultant Hospital Doctors (NCHDs)

Immense reliance on NCHDs to deliver care continues to be a strong feature of the Irish healthcare system. NCHDs work under the supervision of a consultant doctor, however, there is a significant shortage in the number of approved training posts and precarious career training pathways for non-training scheme doctors who are filling growing gaps in the system. Ireland has an excess of NCHDs not in training, relative to specialists. In 2022, there were a similar number of NCHDs in training and not in training.

3. International Medical Graduates (IMGs)

The majority of NCHDs not in training are doctors who have trained overseas, and many have not been able to access specialist training in Ireland. Ireland is highly dependent on international medical graduates (IMGs) and has the fourth highest proportion of foreign educated doctors within OECD member states. Although recent changes to the application of EU/EEA Community Preference to postgraduate medical training programmes by the Postgraduate Medical Training Bodies are to be welcomed, further action is required to improve supervision and working conditions of IMGs. Working conditions and quality of training must be significantly improved if non-EU/EEA doctors are to be encouraged to remain working in Ireland.

4. Workforce Shortage

Hospital Consultants

Consultant posts are not being adequately filled and this impacts on supervision and training for trainees and junior doctors. Sufficient numbers of consultants need to be trained and retained in order to have suitably trained applicants for new and replacement consultant posts. From the Medical Council perspective, if doctors do not receive appropriate education and training, patient and professional safety issues arise.

General Practice

Ireland has an ageing General Practitioner (GP) workforce, and a substantial proportion of GPs are close to retirement age. There is an urgent need for effective action to tackle Ireland's shortage of GPs, which is causing significant challenges when it comes to delivering timely and high-quality patient care. Increasing pressure on GPs is a result of a growing and ageing population, increased patient multi-morbidity, GP retirement, poor morale and burnout.

5. Non-compliance with European Working Time Directive (EWTD)

In 2022, over one-quarter of doctors reported working more than 48 hours on average a week, in contravention of the EWTD. Excessive work hours are demonstrably associated with attrition, stress, burnout and are predictive of adverse event involvement, which can undermine patient safety. Non-compliance with the working time limits set out in the EWTD must be sufficiently addressed and improved.

6. Doctor Attrition

Our research indicates that family and personal issues, lack of training opportunities, inadequate resourcing and work conditions are all reasons for doctor withdrawal. In 2022, there were 1,341 voluntary withdrawals recorded, an increase of 359 on 2021's figure. Ireland's inability to retain experienced and highly competent doctors can potentially impact quality of care.

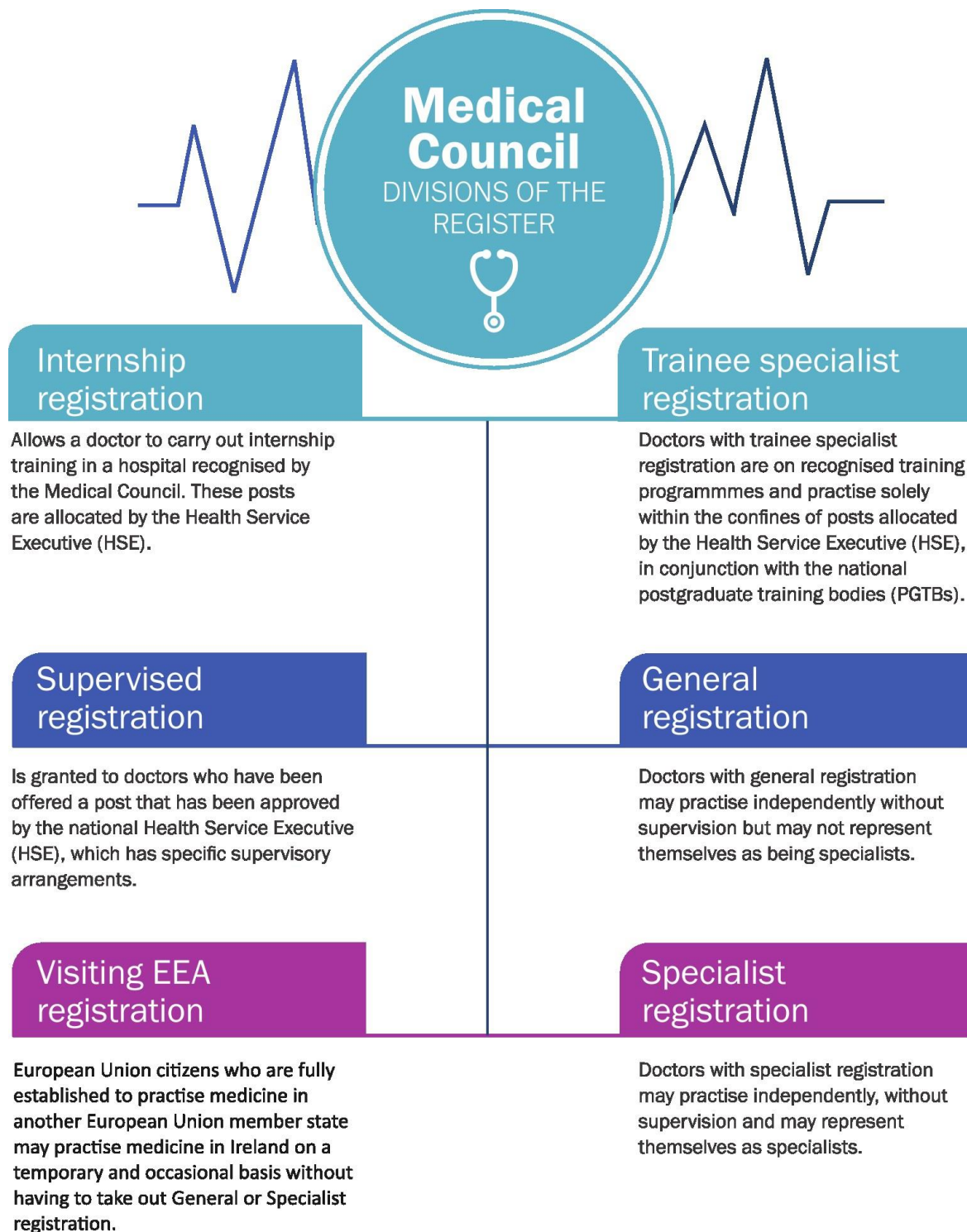
Background

This report presents the analyses of the medical registration data gathered by the Medical Council through its annual registration retention process, from June to November 2022. It also draws on existing registration data to provide a cross-sectional overview of the retained doctors, new entrants, and voluntary withdrawals from the Register in 2022.

The Register is a valid and complete list of doctors who are permitted under Irish law to practise medicine in the State. The cornerstone of the Medical Council's work in protecting the public is establishing and maintaining a Register of doctors. Under Irish law, no one can practise medicine in Ireland unless they are registered as a doctor with the Medical Council. Doctors register in one of six divisions of the Register, depending on the training they have completed, or are currently undertaking.

The six divisions of the Register are as follows:

- Internship registration
- Trainee specialist registration
- Specialist registration
- General registration
- Supervised registration
- Visiting EEA Practitioners registration



Internship registration allows a doctor to carry out internship training in a hospital recognised by the Medical Council. These posts are allocated by the HSE. In accordance with the EC Directive 2005/36/EC, completion of Basic Medical Training is required for registration beyond this. This is the combination of both a Basic Medical Qualification (BMQ), obtained on graduation from a medical degree programme, with an accompanying certificate, if specified. In Ireland, the accompanying certificate is the certificate of experience, gained through satisfactory completion of an internship. All doctors are required to maintain their Professional Competence on as set out by the Medical Council. Throughout the year, doctors enter and leave the Medical Council's Register. Because the Medical Council's Register is a valid and complete list of doctors who are permitted under Irish law to practise medicine in the State, it is a comprehensive source of medical workforce intelligence.

Methods

Three different datasets are analysed in this report, and more details on each can be found below:

1. First registered doctors
2. Doctors who complete retention
3. Doctors who voluntarily withdraw from the register.

First registered doctors

Information on first registered doctors comes from our data management system PRISM. This data covers all doctors who registered for the first time with the Medical Council between 1st January and 31st December 2022.

Notably, a very small number of doctors may register for the first time and leave the register in the same year. In 2022, 94.5% of doctors who registered for the first time remained active at the end of the year. Information available for these doctors is limited to demographic details including age, sex, and category of BMQ. For the first time, there are now five categories of registration:

- Category 1 – Graduates of Irish Medical School,
- Category 2 – Graduates of a medical school in the EU and are EU nationals,
- Category 3 – Graduates from a medical school in the EU and are not EU nationals,
- Category 4 – International graduates from a medical school outside of Ireland, the UK and the EU; and
- Category 4 (UK) – International graduates from a medical school in the UK. This extra Category 4 (UK) is due to Brexit and all doctors who graduate from a medical school in the UK since Brexit fall into this category. For this reason, this category is relatively small as doctors who graduated from a medical school in the UK prior to Brexit would fall into category 2 or 3 as appropriate.

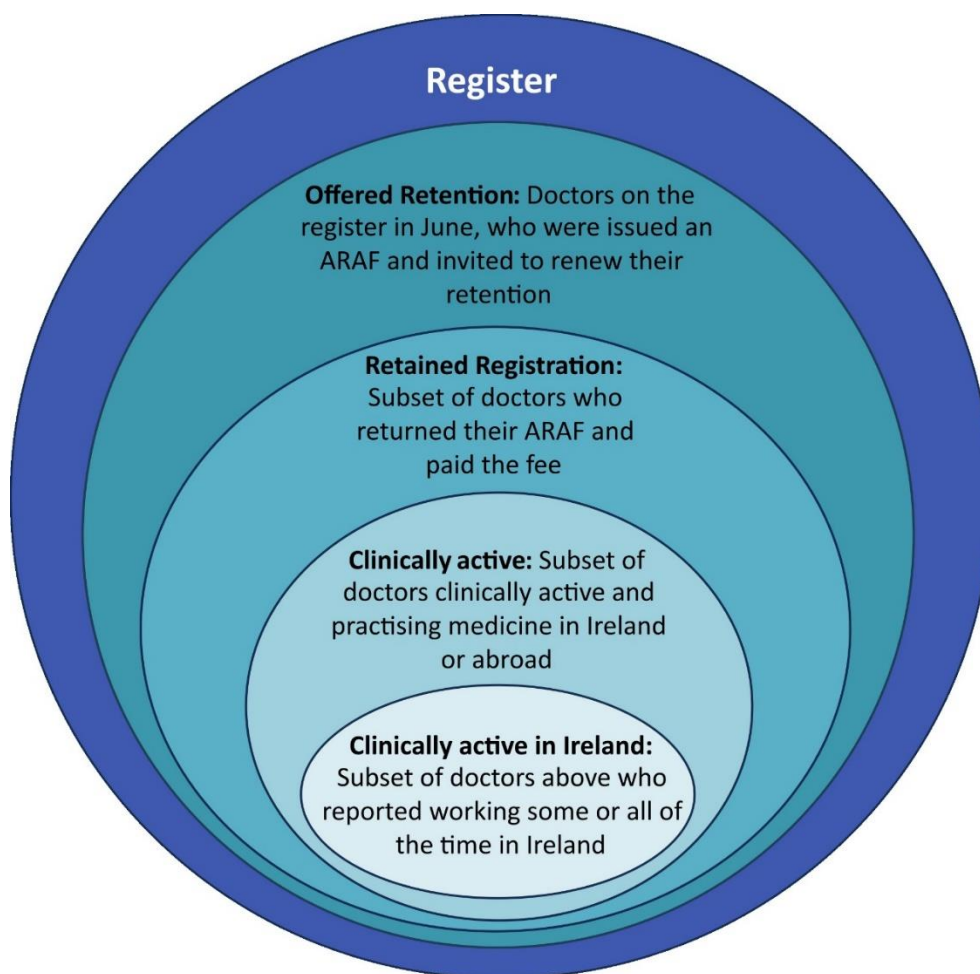
Doctors who complete retention

All doctors are obligated to renew their registration annually if practising medicine, and failure to complete this process can ultimately result in a doctor's name being removed from the Register, following a warning and notification of potential removal. All doctors who are active on the Register as of the 30th April each year are invited to undergo retention. The annual retention process is available through each doctor's personal online registration account. Doctors must submit a completed annual retention application form (ARAF) along with the relevant fee. The ARAF has several compulsory questions where doctors must make declarations about their current position. Since 2012, the Medical Council has included a set of questions to collect and analyse data on registered doctors' work

practices to inform strategic workforce planning, regulatory operations and development. These questions are voluntary but have a high response rate. These data have been analysed against basic information about the doctors' age, gender, country of BMQ and divisional credentials.

Doctors who retain registration are asked if they have practised medicine in the previous 12 months and where (in Republic of Ireland only, both within and outside the Republic of Ireland or outside the Republic of Ireland only). These two questions were used to filter our data to identify doctors who were clinically active in Ireland in 2022. Notably, these questions are retrospective and while it is likely if a doctor practised in the previous 12 months, they would still be clinically active during the retention period, a small number may have subsequently stopped practising medicine. Furthermore, it is not compulsory for doctors to answer this question and as such the figures may be very slightly under the true number. See Figure 1 below for more detail.

Figure 1. Illustration Of How The Different Datapoints Within The Clinically Active Folder Are Filtered



Doctors who do not complete retention may voluntarily withdraw or be removed from the Register due to failure to fulfil retention requirements. Data on voluntary withdrawals is presented in this report.

Doctors who have just completed their first year of postgraduate training or ‘internship’ year, are not required to complete the annual retention process but rather they apply to the Medical Council to transfer registration. Doctors who hold Visiting EEA registration are similarly not required to apply to retain registration with the Medical Council as this is time limited.

This report builds on previous Workforce Intelligence Reports (WIRs), identifying patterns and recurring issues. The Register of Medical Practitioners is a “living” database as doctors are entered on, removed from, or transferred between the divisions of the Register. For this reason, comparison between reports based on registration data must take account of this “living” nature of the database. For example, the calendar year-end totals reflect any registration activity, including doctors entering or leaving the Register, between June and December 2022. Totals taken at year-end differ slightly from the ‘retention of registration’ data, which was collected between June and November 2022.

Voluntary Withdrawals

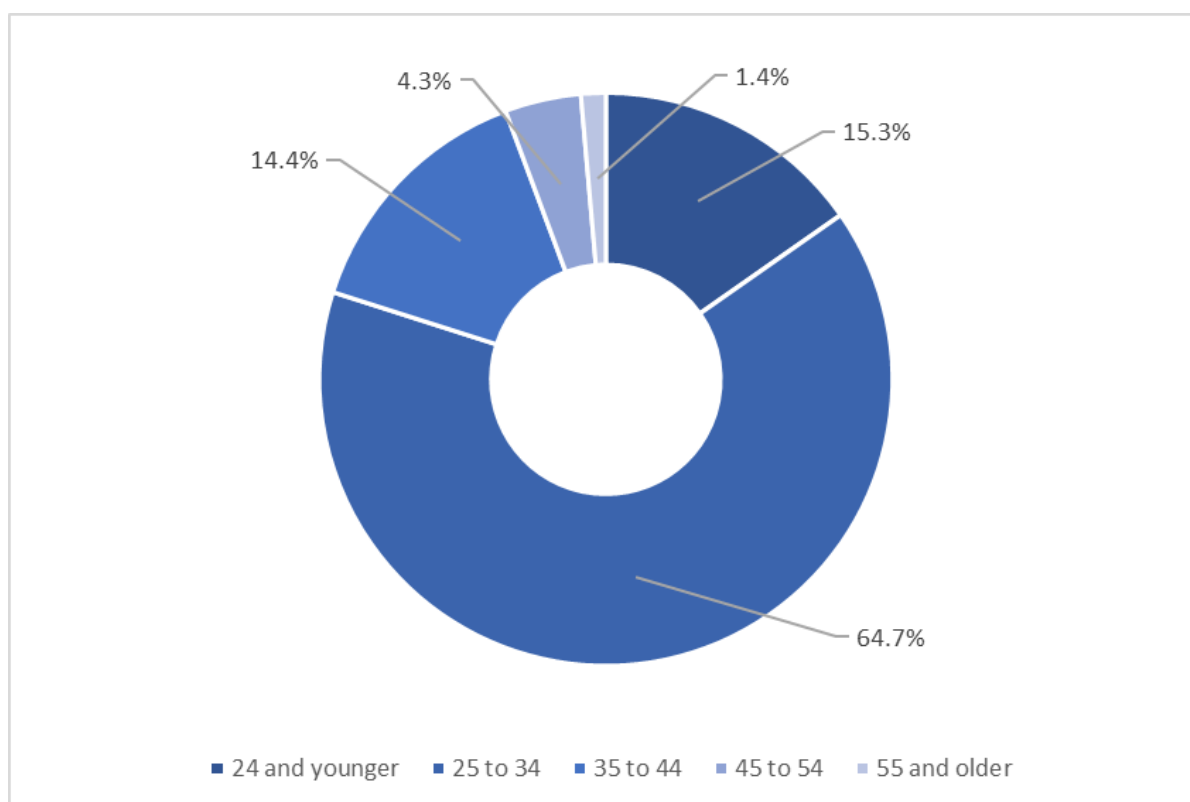
Doctors can also voluntarily withdraw from the Register if they no longer wish to practise medicine in Ireland. Doctors who voluntarily withdrew in 2022 cover the period from the 1st of January 2022 to the 31st of December 2022. All doctors who voluntarily withdraw are sent a survey to complete to determine the primary reason for withdrawing. They are also given the opportunity to provide qualitative feedback. Demographic and divisional details of the doctor are derived from our internal data management system PRISM.

Results

First Registered Doctors

Between the 1st of January and the 31st of December 2022, 3,008 doctors registered for the first time with the Medical Council. The mean age for this cohort was 30.6 years (SD = 7.1), and more details on the age categories are shown in Figure 2.

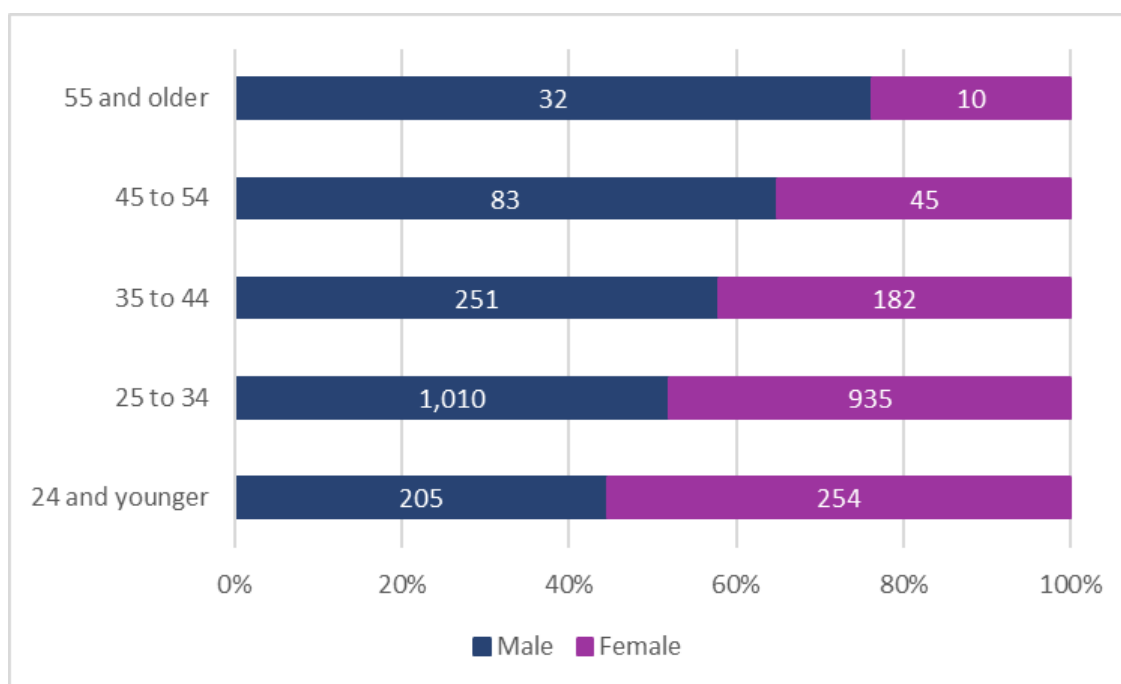
Figure 2. Age Profile Of First Registered Doctors



**Age of one doctor is missing from the above graph.*

The gender breakdown was evenly split, with slightly more males (N = 1,581, 52.6%) than females (N = 1,427, 47.4%) registering for the first time. Notably, the ratio of females to males is greater in the youngest cohort of doctors. Figure 3 shows gender by age group.

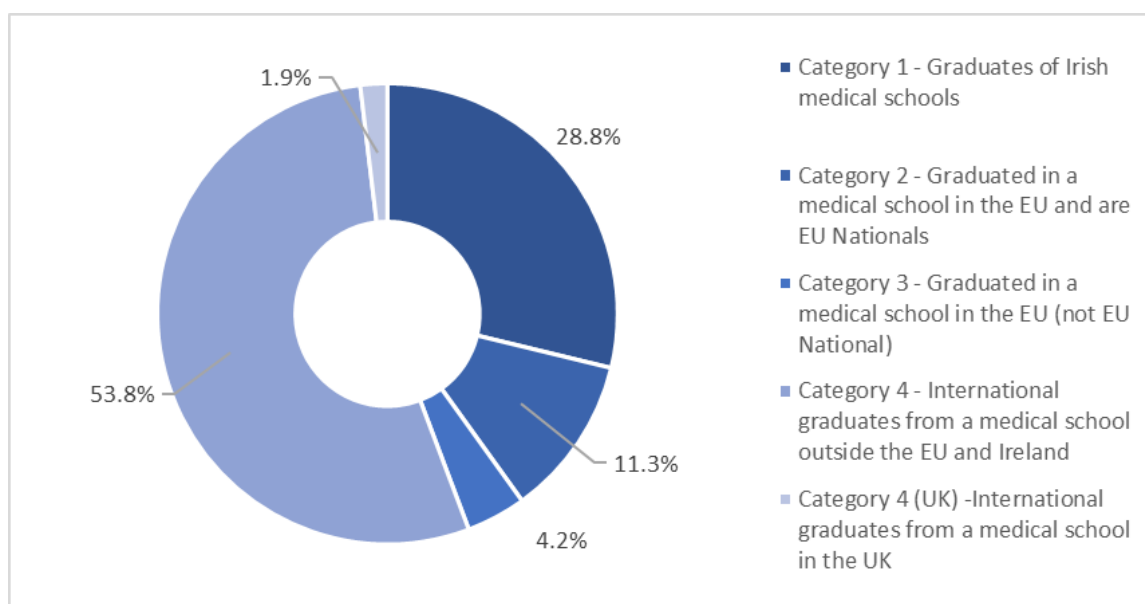
Figure 3. Gender By Age Group



** Age of one doctor is missing from the above graph.*

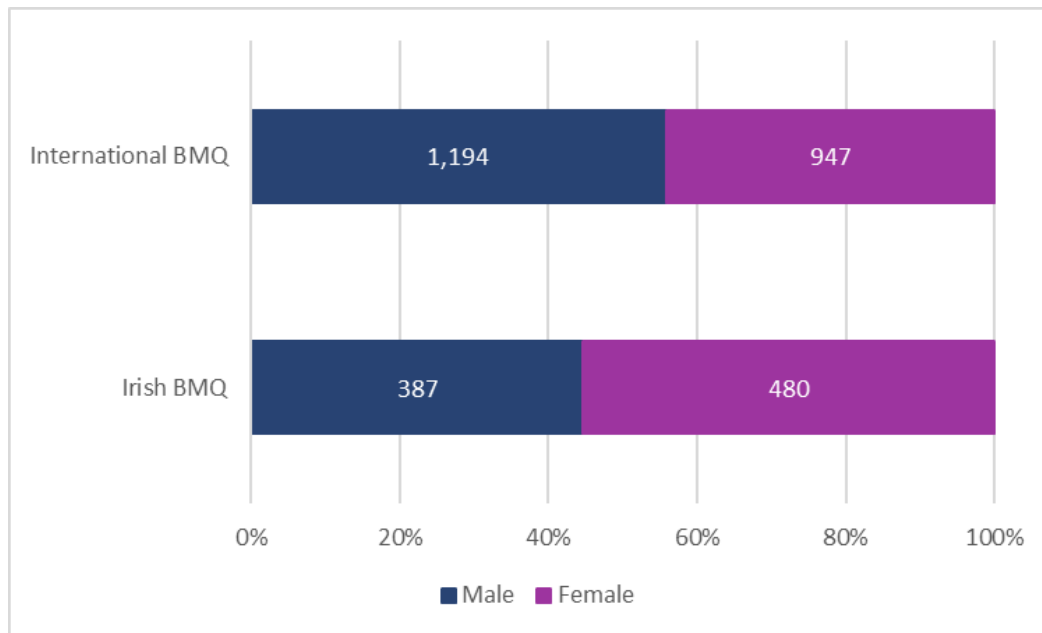
In relation to the BMQ of newly registered doctors, 867 (N = 28.8%) had Irish qualifications, and 2,141 (71.2%) obtained their qualifications abroad. Figure 4 shows a more detailed breakdown of BMQ categories.

Figure 4. BMQ Category



BMQ was divided into two groups; category 1: Irish graduates and category 2-4 (UK) international graduates. There were more females than males among Irish graduates and more males than females among international graduates. Figure 5 below shows this in more detail.

Figure 5. Gender By BMQ



Of the 3,008 doctors first registered in 2022, 94.5% (N = 2,843) remained active on the register by the end of the year.

Doctors Offered Retention and Retaining

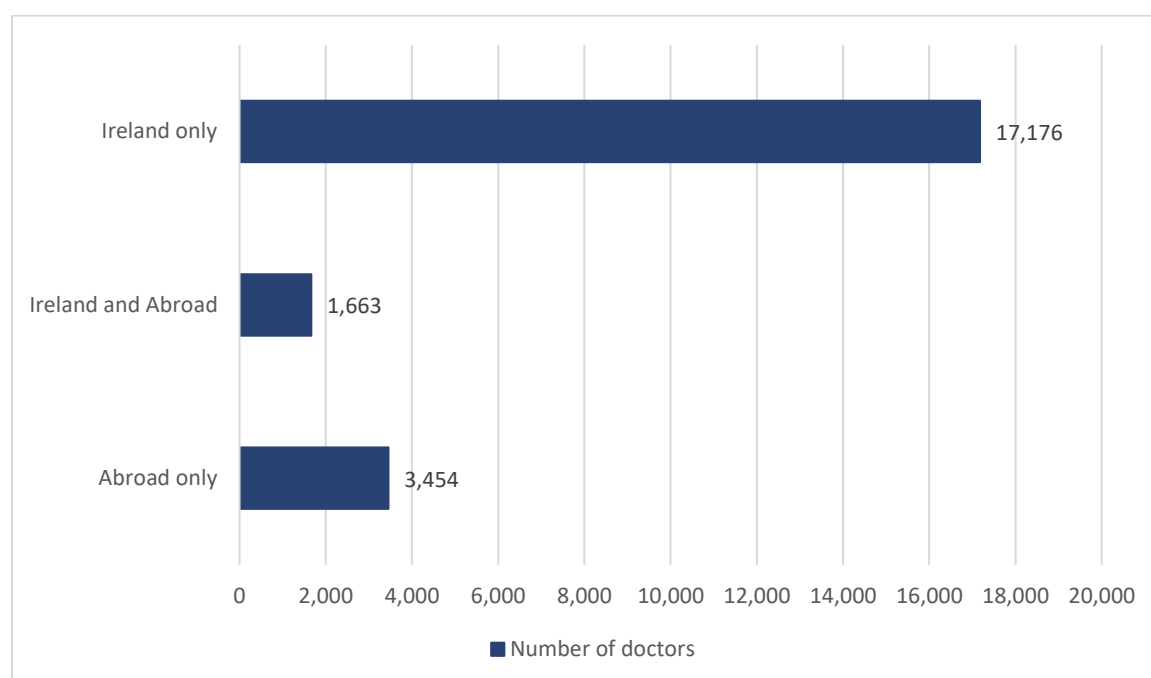
In June 2022, 23,686 doctors on the register of Medical Practitioners in Ireland were invited to retain their registration with the Medical Council. Of these doctors, 23,108 (97.5%) doctors chose to retain their registration. These doctors were aged between 23 and 94 years, with an average age of 44.1 years (SD = 12.2 years). Just over half of these doctors were male (N = 12,747, 55.2%). Most of these doctors were graduates from a medical school in Ireland (N = 13,037, 56.4%), and were on the specialist division of the register (N = 11,139, 48.2%). Further details on the gender, Basic Medical Qualification (BMQ) and division of doctors retaining their registration can be found in Appendix A.

Clinically Active Doctors

All doctors who retained their place on the register were asked if they practised medicine in the past 12 months. If doctors answered 'yes' to this question we defined them as being **clinically active**. In 2022, 22,317 (96.6%) reported practising medicine in the past 12 months, 696 (3.0%) reported not practising medicine in the past 12 months and 95 (0.4%) did not answer this question, meaning that it was not possible to determine if they had practised medicine in the past 12 months.

Of those reporting that they were clinically active, 17,176 (77.0%) doctors practised in Ireland only, 1,663 (7.5%) practised in Ireland and outside of Ireland, 3,454 (15.5%) practised outside of Ireland only, and 24 (0.1%) did not provide detail on where they were practising (see Figure 6). This means that at least 15.5% of those who retained their registration were not practising medicine in Ireland.

Figure 6. Breakdown Of Where Clinically Active Doctors Were Practising In 2022



** The 24 doctors who did not answer this question are excluded from this figure and subsequent analysis.*

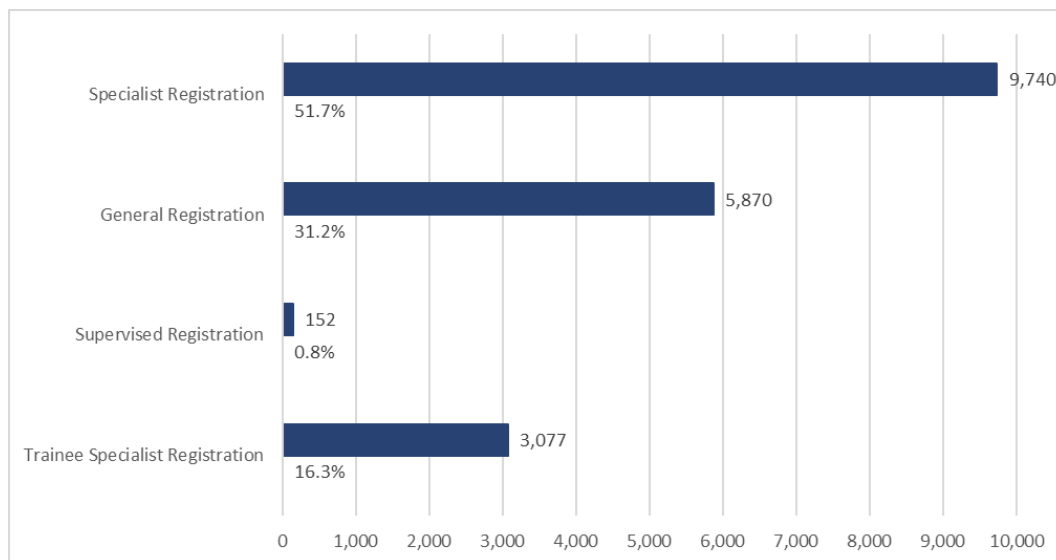
Overview of Clinically Active Doctors in Ireland

The sections which follow focus on doctors reporting to be working in Ireland, all or some of the time (N = 18,839). This represented 81.5% of doctors who retained their registration.

Demographics

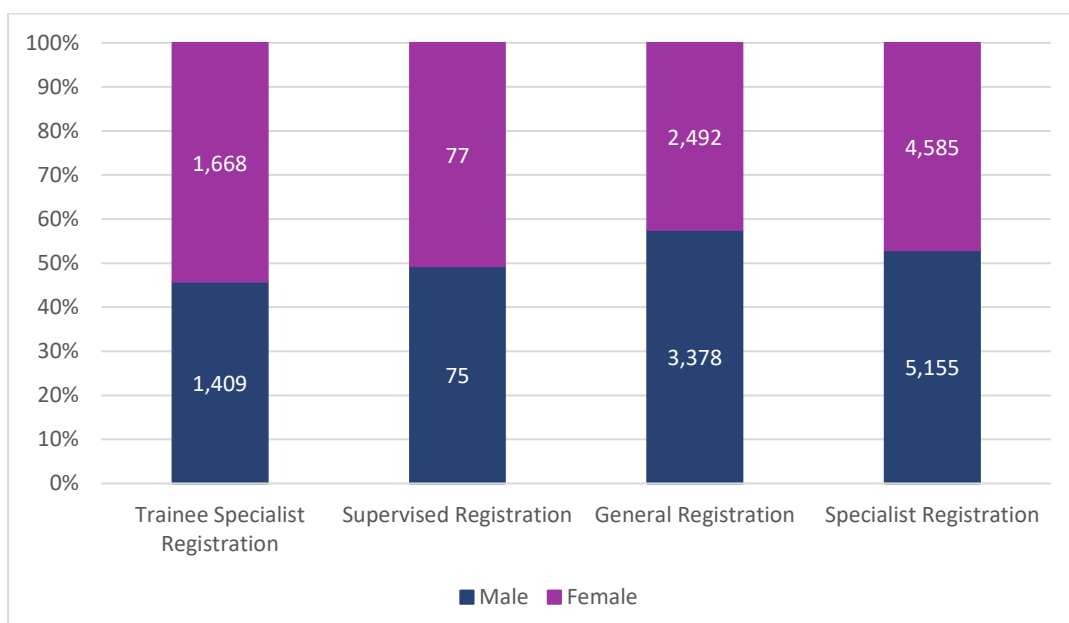
The division with the largest proportion of clinically active doctors was the Specialist Division, with 9,740 members (51.7%) (Figure 7).

Figure 7. Divisions Of Register



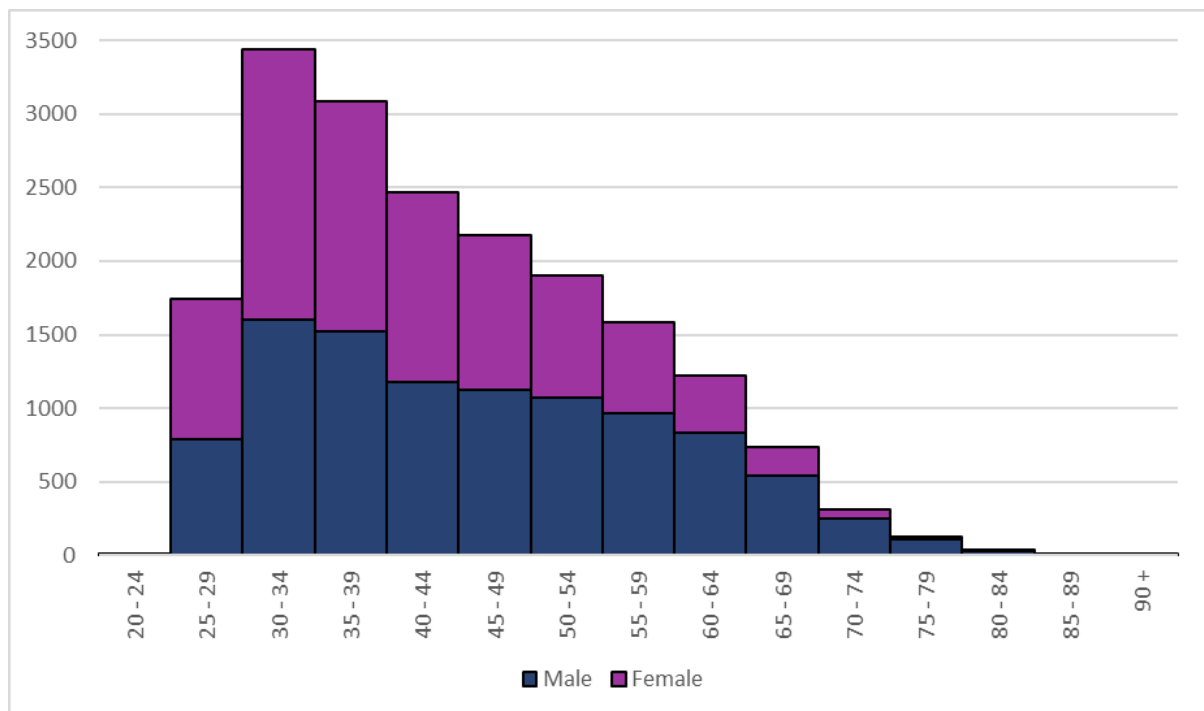
The gender breakdown of clinically active doctors in Ireland consisted of 10,017 (53.2%) males and 8,822 (46.8%) females. Figure 8 below illustrates gender by division.

Figure 8. Gender By Division



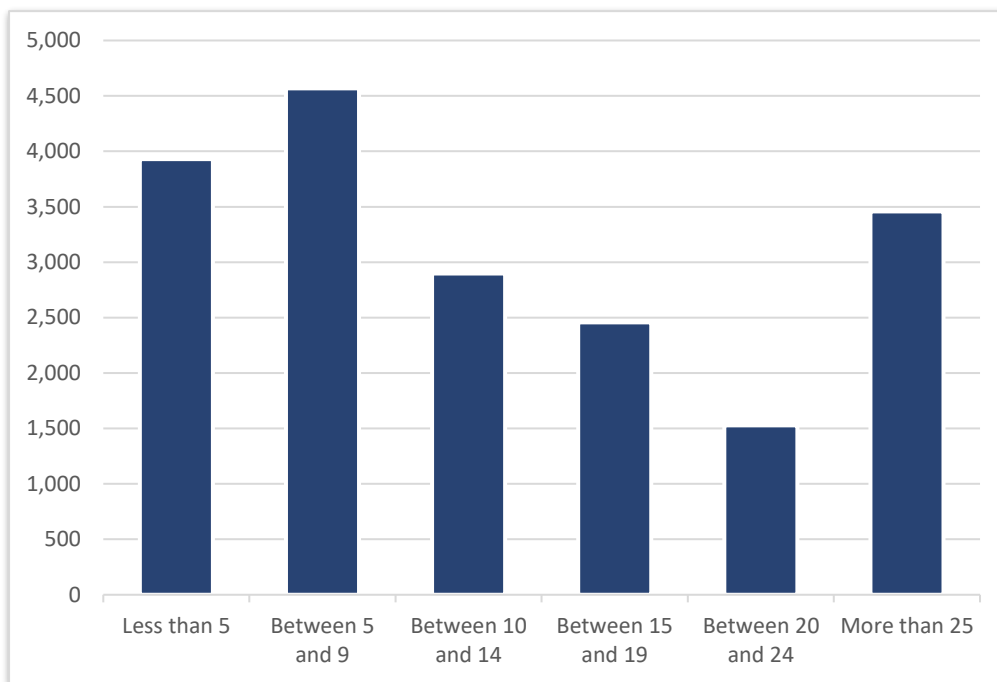
The mean age of clinically active doctors in Ireland was 43.9 (SD=12.1) years. A total of 21.3% were aged 55 and above, and therefore approaching the typical retirement age. Figure 9 below illustrates a histogram of age by gender. It can be observed, the distribution of males and females tends to be split more evenly in younger cohorts, whereas the majority of older doctors are male.

Figure 9. Histogram Of Age By Gender



Doctors' years of practice was calculated from their date of first registration with the Medical Council. This estimate does not exclude any time doctors might have temporarily withdrawn from the Register, or practiced outside the State, but offers a rough approximation of how many years of practice they may have. Clinically active doctors had practised for an average of 14.1 years (SD = 11.1), however, some doctors had been practising for up to 42 years. This average is also reflective of the age distribution of doctors. Figure 10 shows a breakdown of years of practice.

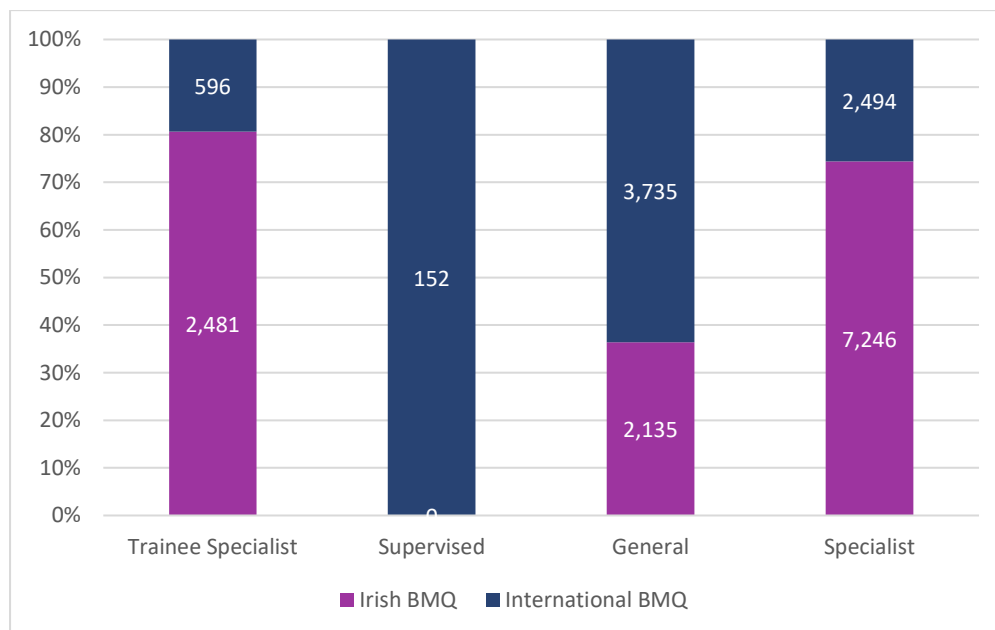
Figure 10. Clinically Active Doctors' Years Of Practice



In relation to BMQs, 11,862 (63.0%) clinically active doctors had obtained their qualification in Ireland, 2,582 (13.7%) in the EU or UK and 4,395 (23.3%) from a medical school outside of Ireland, the EU or the UK.

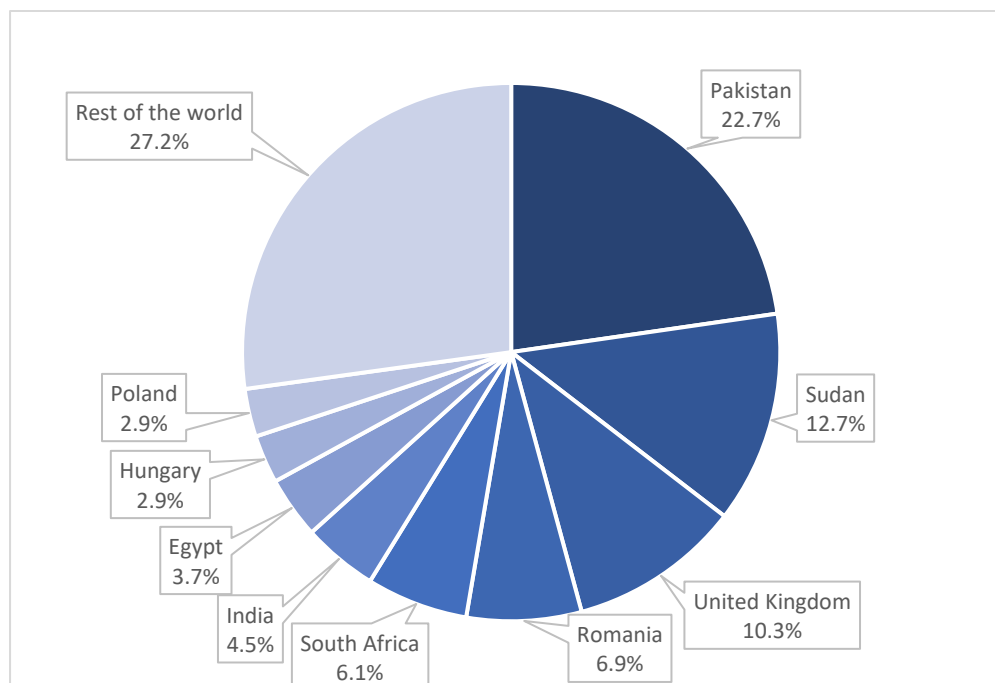
BMQ was split into two groups, those who graduated from an Irish Medical School and those who graduated from an EU, UK or an international medical school beyond this. As can be observed from Figure 11, the majority of those enrolled in the Trainee Specialist Division (N = 2,481, 80.6%) and the Specialist Division (N = 7,246, 74.4%) were Irish BMQ holders, while almost two thirds (N = 3,735, 63.6%) of doctors in the General Division had BMQs from a medical school outside of Ireland.

Figure 11. Division By BMQ



Considering those who received their BMQs abroad, Pakistan, Sudan, and the United Kingdom were the three most common countries where international graduates trained, accounting for 3,191 doctors (45.8%) in this cohort. More information can be found in Figure 12 and Table B1 in the Appendix B.

Figure 12. Country Of Qualification For International BMQ Holders



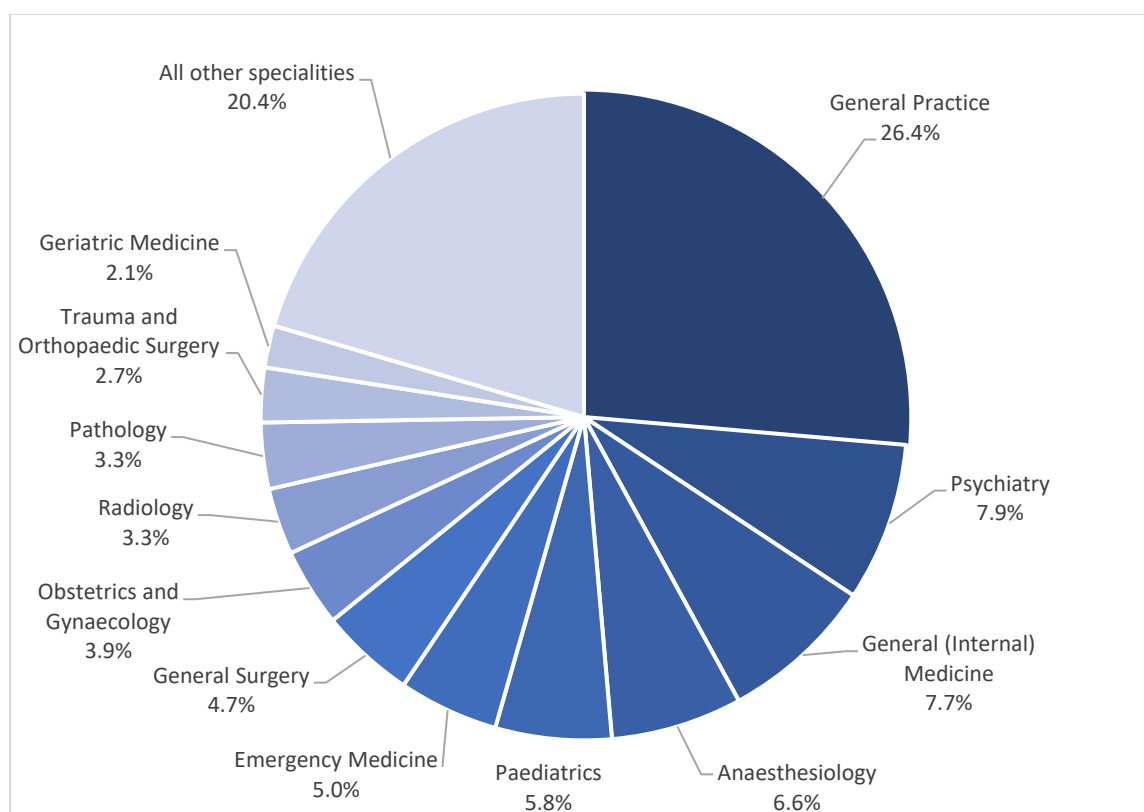
**this information was not available for 9 doctors and these are excluded from this figure.*

Employment and practice

In 2022, 4,953 (26.4%) clinically active doctors reported their current or most recent practice of medicine in the specialty of General Practice, followed by Psychiatry (N = 1,490, 7.9%), General (Internal) Medicine (N = 1,453, 7.7%), and Anaesthesiology (N = 1,234, 6.6%).⁴

Figure 13 illustrates the most common current area of practice reported (those exceeding 2%), and Figure 14 displays this among those on the specialist division of the register. A full breakdown of current area of practice can be found in Tables C1, and C2, in Appendix C.

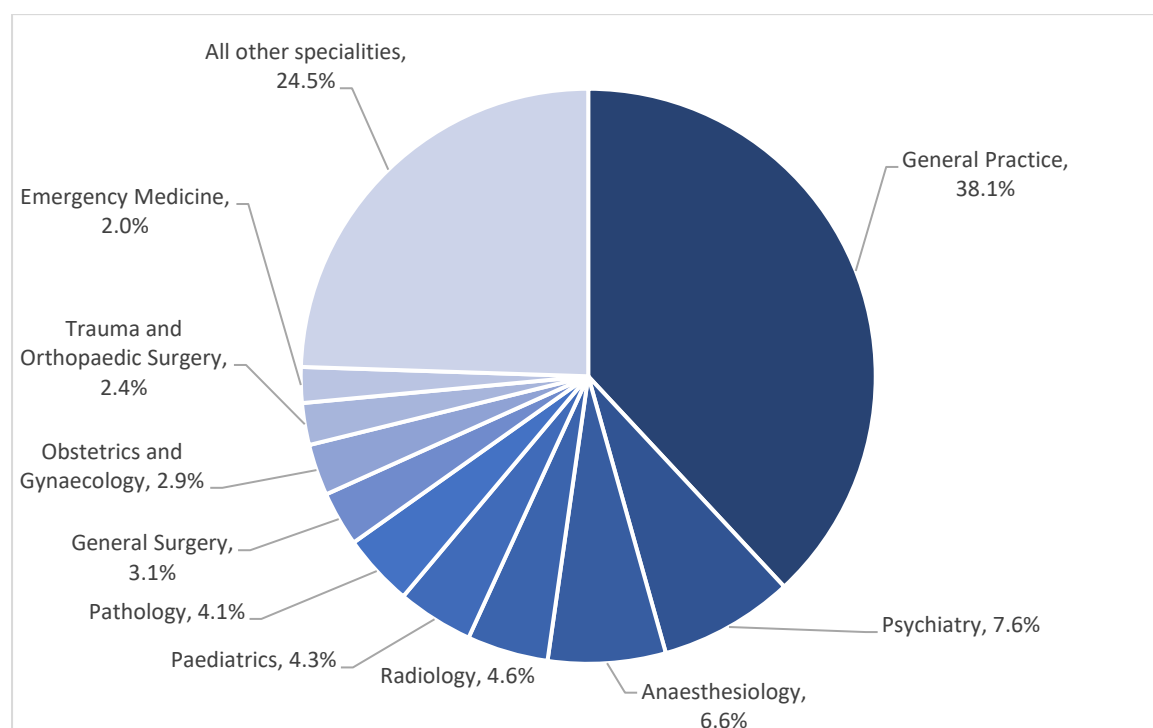
Figure 13. Most Common Areas Of Practice Of Clinically Active Doctors



**60 doctors did not provide information on their current area of practice and are excluded from this figure.*

⁴ For the purpose of this consolidated report subspecialties were grouped together. Psychiatry includes those working in psychiatry generally, child and adolescent psychiatry, psychiatry of old age and psychiatry of a learning disability. Paediatrics includes paediatrics generally, neonatology and paediatric cardiology. Radiology includes radiology generally and radiation oncology. Pathology includes chemical pathology, haematology (Clinical & Laboratory), Histopathology, Immunology (Clinical & Laboratory), Microbiology, and Neuropathology. Larger groups of medicine and surgery are not grouped here.

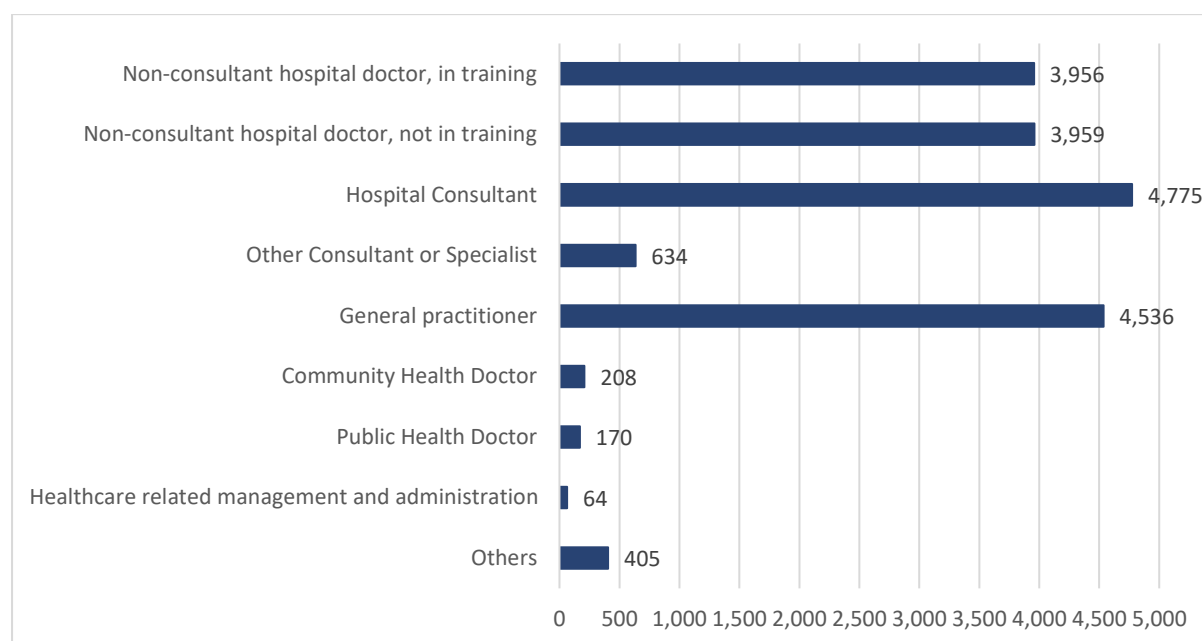
Figure 14. Most Common Areas Of Practice For Clinically Active Specialists



**22 doctors on the specialsit division did not provide information on their area of practice and are excluded from this figure.*

As it can be observed in Figure 15, the most common self-reported employment role among clinically active doctors in Ireland was hospital consultants (N = 4,775, 25.5%), closely followed by General Practitioner's (N = 4,536, 24.2%).

Figure 15. Self-Reported Role



**132 doctors did not provide information on self-reported role and are excluded from this figure*

One-third (33.6%, N = 1,605) of hospital consultants and 46.1% (N = 292) of other consultants and specialists were aged 55 years or over, and therefore likely to retire in the coming 10 years. A full breakdown of self-reported role by age, can be found in Table D1, in Appendix D.

When looking at the gender among these roles certain roles are closely split among males and females, for example, 51.4% of other consultants or specialists were male and 48.6% were female. Similarly, among general practitioners 47.4% were male and 52.6% were female. However, the proportion of males to females varies for some of the other categories. For example, there was a higher proportion of males than females working as hospital consultants and there were more females than males working as a NCHDs in training. Figure 16 shows a series of population pyramids (age categories and gender) for NCHD's in training, NCHD's not in training, hospital consultants, and general practitioners.

In the context of BMQs among self-reported employment roles, this translated into three quarters of NCHDs not in a training holding international BMQs (N = 3,003, 75.9%) and a similar proportion of NCHDs in training holding Irish BMQs (N = 3,030, 76.6%). Figure 17 shows a series of population pyramids by age and BMQ for NCHD's in training, NCHD's not in training, hospital consultants, and general practitioners.

Figure 16. Population Pyramids (Age And Gender) Of Some Of The Self-Reported Roles Among Clinically Active Doctors

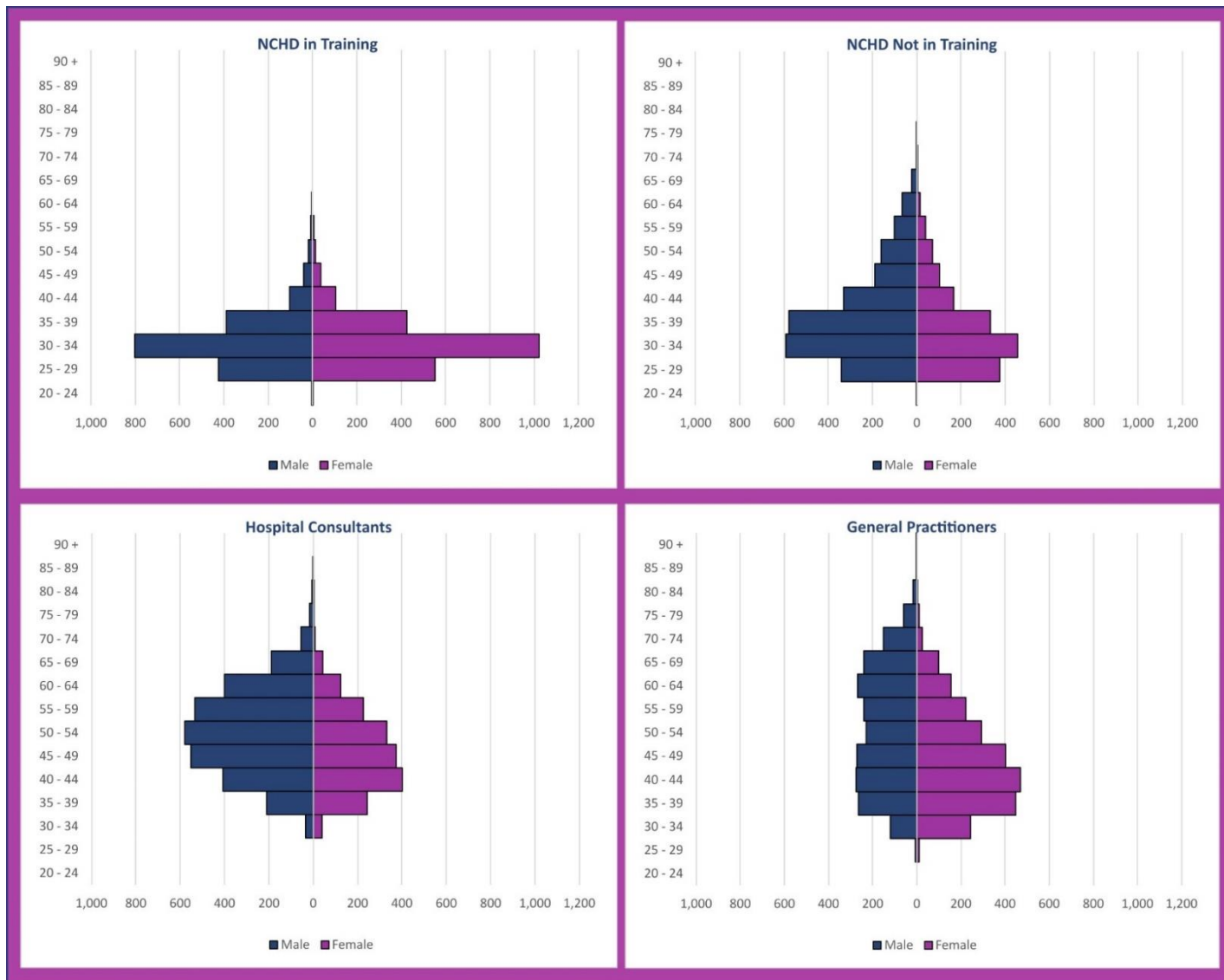
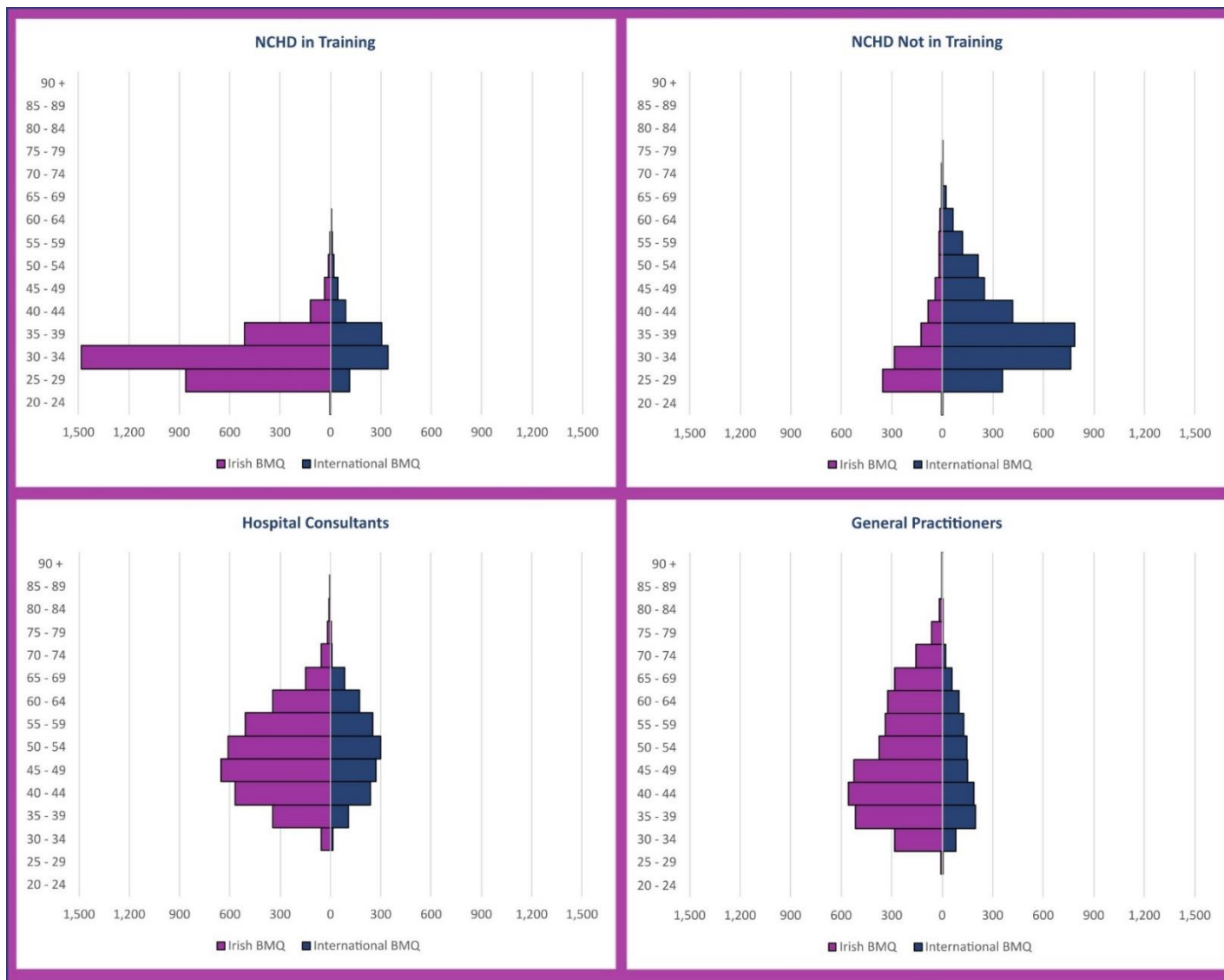


Figure 17. Population Pyramids (Age And BMQ) Of Some Of The Self-Reported Roles Among Clinically Active Doctors



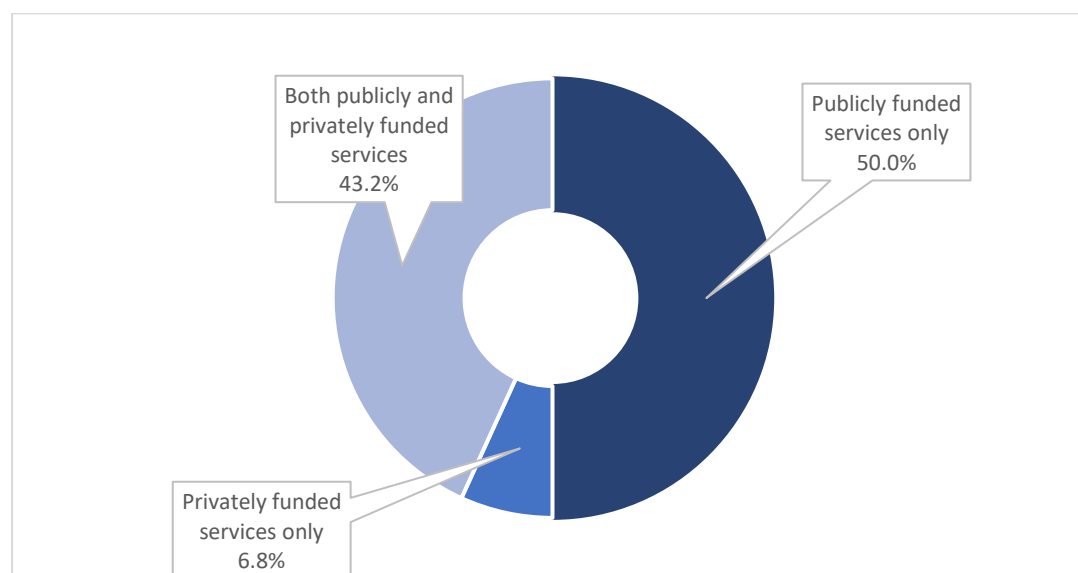
As indicated in Table 1, almost two-thirds reported training other doctors as part of their role. However, just under half of those said this was not a formal responsibility within their role (N = 5,945, 31.6%).

Table 1. Self-Reported Trainer Role

	N	%
Yes, it's part of my role to train other doctors	6,090	32.3%
Yes, but it's not a formal part of my role to train other doctors	5,945	31.6%
No, I do not train other doctors	6,804	36.1%
Total	18,839	100.0%

Data on the type of service provided (publicly funded, private, or both) was self-reported by 17,604 clinically active doctors in Ireland. Half of all clinically active doctors in Ireland self-reported providing publicly funded services only (N = 8,810, 50.0%), while a further 43.2% (N = 7,598) reported providing both publicly and privately funded services, and only 6.8% (N = 1,196) reported providing privately funded services only (see Figure 18).

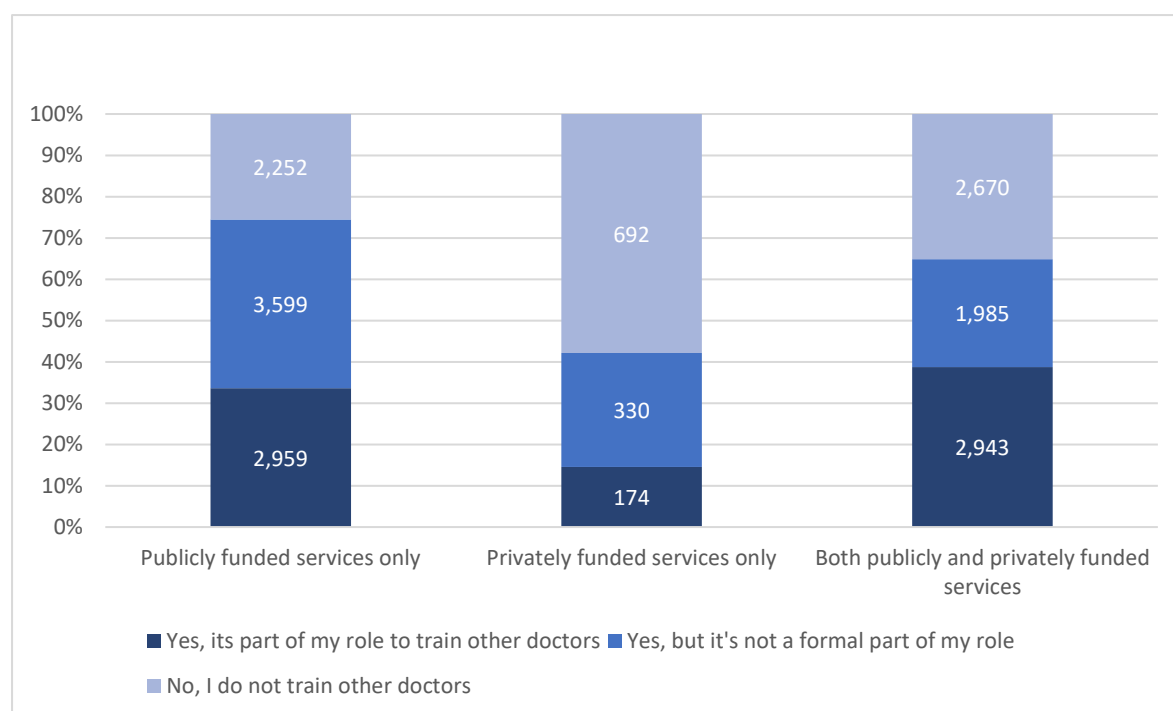
Figure 18. Service Provision Among Clinically Active Doctors In Ireland



**1,235 doctors did not answer this question and are excluded from this figure and the above analysis.*

Furthermore, when service provision type is broken down by self-reported trainer role (Figure 19), more doctors reported training duties when working in publicly funded services only (whether this is part of their role or not) (74.5%) than when employed exclusively by the private sector (42.1%).

Figure 19. Service Provision By Self-Reported Trainer Role



**1,235 doctors did not provide information on type of service provision and are excluded from this figure*

Working Hours

Doctors were asked to report on number of hours they worked on an average week and 99.3% (N = 18,704) doctors provided this detail. Of these, 28.6% (N = 5,347) reported working more than 48 hours per week, in contravention of the European Working Time Directive (EWTD). In addition to this, almost a third (N = 6,145, 32.9%) reported working 40 to 48 hours per week (Table 2).

Table 2. Hours Worked Per Week

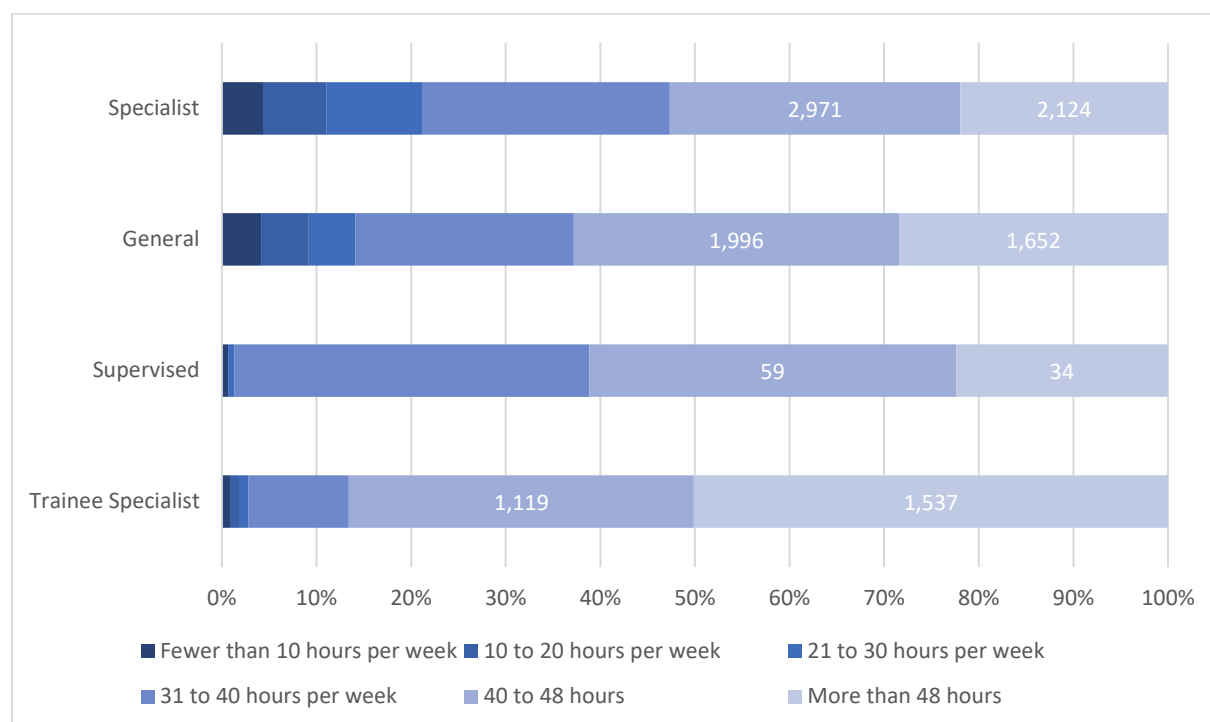
	N	%
Fewer than 10 hours per week	688	3.7%
10 to 20 hours per week	974	5.2%
21 to 30 hours per week	1,300	7.0%
31 to 40 hours per week	4,250	22.7%
40 to 48 hours	6,145	32.9%
More than 48 hours	5,347	28.6%
Total	18,704	100.0%

**135 doctors did not provide information on hours worked and are excluded from this table*

When looking at the percentage of different divisions working in contravention of the EWTD over half of all trainee specialists report working more than 48 hours per week (1,537, 50.1%). This is in

comparison to 28.4% (N = 1,652) of those on the general division, 22.4% (N = 34) of those on the supervised division and 22.0% (N = 2,124) of those on the specialist division.

Figure 20. Working Hours By Division



**135 doctors did not provide information on hours worked and are excluded from this figure.*

A higher proportion of doctors with an Irish BMQ (N = 3,692, 31.3%) were working more than 48 hours per week in comparison to those with a BMQ from outside of Ireland (N = 1,655, 24.0%). However, when looking at those working 40-48 hours the opposite trend was observed.

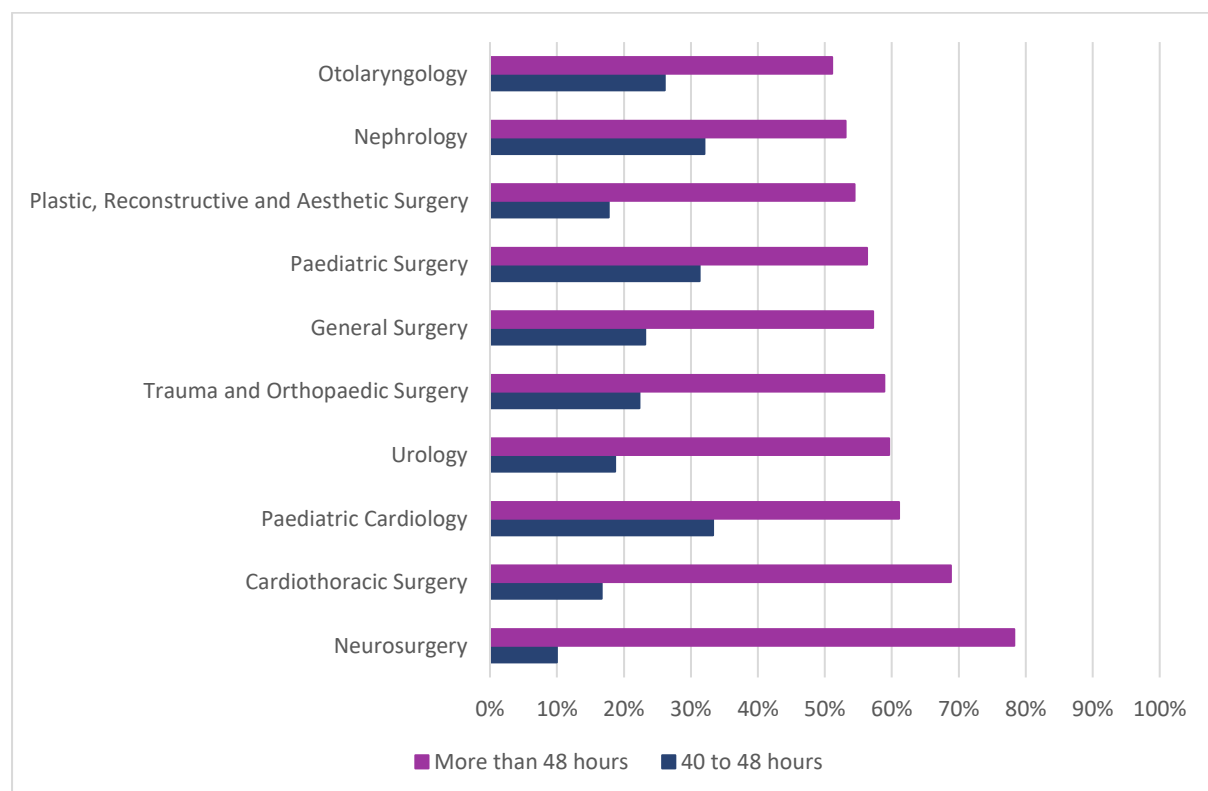
Table 3. The Breakdown Of Doctors With An Irish Or International BMQ By Hours Worked Per Week.

	Irish BMQ		International BMQ	
	N	%	N	%
< 10 hours	521	4.4%	167	2.4%
10 – 20 hours	735	6.2%	239	3.5%
21-30 hours	970	8.2%	330	4.8%
31-40 hours	2,273	19.2%	1,977	28.7%
40–48 hours	3,617	30.6%	2,528	36.7%
> 48 hours	3,692	31.3%	1,655	24.0%
Total	11,808	100.0%	6,896	100%

**135 doctors did not provide information on hours worked and are excluded from this table*

Higher percentages of doctors working in certain areas of practice reported working in contravention of the EWTD. For example, nearly 4 out of 5 doctors working in the area of neurosurgery reported working more than 48 hours per week (N = 60, 78.3%). In ten areas of practice over half of doctors in the specialty reported working more than 48 hours in an average week. These are presented in Figure 21 below.

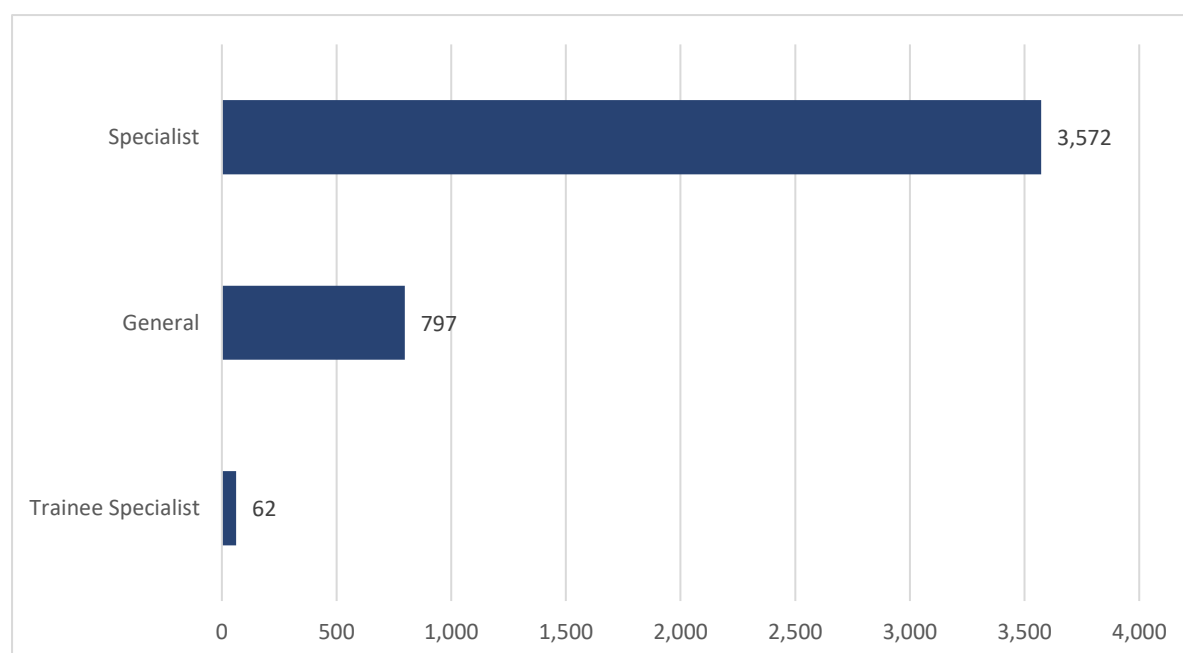
Figure 21. Hours Worked In The 10 Current Areas Of Practice Where More Than Half Of Doctors Were Working In Contravention With The EWTD



Spotlight: General Practitioners working in General Practice

To have the most complete picture of mainstream GPs working in general practice, only doctors who selected both General Practice for their current area of practise and GP for their self-reported employment role are included in following analysis. The Division with the largest proportion of doctors was the Specialist Division, with 3,572 members (Figure 22).

Figure 22. Division Of Register Of Doctors Working In General Practice



There were slightly more females (N = 2,333, 52.7%) than males (N = 2,098, 47.3%) in this area and the mean age of this cohort was 49.5 (SD = 11.8) years. Of this cohort, 13.4% (N = 595) were aged 65 and over and therefore likely to enter retirement within the next 5 years. A further 19.7% (N = 873) were aged between 55 and 64 and are therefore likely to retire in the next 10 years. This suggests that nearly one-third of clinically active GPs working in general practice in Ireland were aged 55 and above.

The majority of doctors had an Irish BMQ (N = 3,390, 76.5%) and had been practising medicine for up to 42 years (M = 19.9 years, SD = 11.6). Among clinically active GPs in general practice, 11.7% (N = 517) reported working more than 48 hours per week, in contravention with the EWTD. A further 25.7% (N = 1,134) were working between 40 and 48 hours per week.

Table 4. Hours Worked Per Week Among GPs In General Practice

	N	%
Fewer than 10 hours per week	266	6.0%
10 to 20 hours per week	431	9.8%
21 to 30 hours per week	713	16.1%
31 to 40 hours per week	1,359	30.7%
40 to 48 hours	1,134	25.7%
More than 48 hours	517	11.7%
Total	4,420	100.0%

**11 doctors (0.2%) did not provide information on hours worked and are excluded from this table.*

Based on the total figures of those in general practice working as a GP there were a total of 0.86 GPs (on general, specialist, or trainee specialist division) and 0.69 specialists GPs per 1000 people living in the country.⁵

County of practice was provided by 4,375 (98.7%) of clinically active GPs working in general practice and more detail on the breakdown can be observed in the table below.

Table 5. Self-Reported County Of Practice Among Clinically Active GPs In General Practice

County	N	%
Carlow	52	1.2
Cavan	57	1.3
Claire	86	2.0
Cork	564	12.9
Donegal	155	3.5
Dublin	1,334	30.5
Galway	283	6.5
Kerry	131	3.0
Kildare	162	3.7
Kilkenny	65	1.5
Laois	63	1.4
Leitrim	21	0.5
Limerick	211	4.8
Longford	37	0.8
Louth	109	2.5
Mayo	106	2.4
Meath	118	2.7
Monaghan	32	0.7
Offaly	60	1.4
Roscommon	51	1.2
Sligo	75	1.7
Tipperary	124	2.8
Waterford	124	2.8
Westmeath	95	2.2
Wexford	133	3.0
Wicklow	127	2.9
Total	4,375	100

* 56 GPs in General Practice did not provide information on county of practice.

⁵ The CSO figures for total population in Ireland were used for these calculations.

Doctors who Retain Registration but are not Practising

In 2022, 696 (3.0%) of those who retained their place on the register reported not practising medicine in the last 12 months.

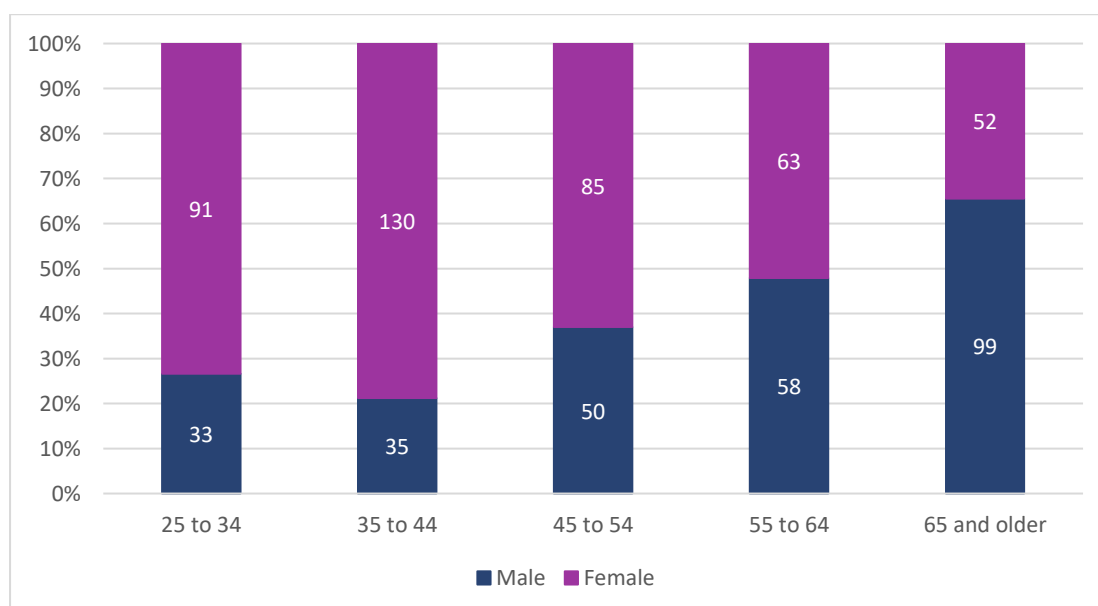
There were more females (N = 421, 60.5%) than males (N = 275, 39.5%) retaining their place and not practising. The majority of these doctors were registered on the general division (N = 394, 56.6%), followed by those on the specialist (N = 275, 39.5%), and the trainee specialist (N = 26, 3.7%). These doctors were aged between 25 and 86 (M = 50.4, SD = 15.1) years, with the majority of respondents aged between 35 and 44 years (N = 165, 23.7%). However, the distribution between age groups was very similar.

Table 6. Age Categories Of Doctors Retaining But Not Practising

Age Category	N	%
34 and younger	124	17.8%
35 to 44	165	23.7%
45 to 54	135	19.4%
55 to 64	121	17.4%
65 and older	151	21.7%
Total	696	100.0%

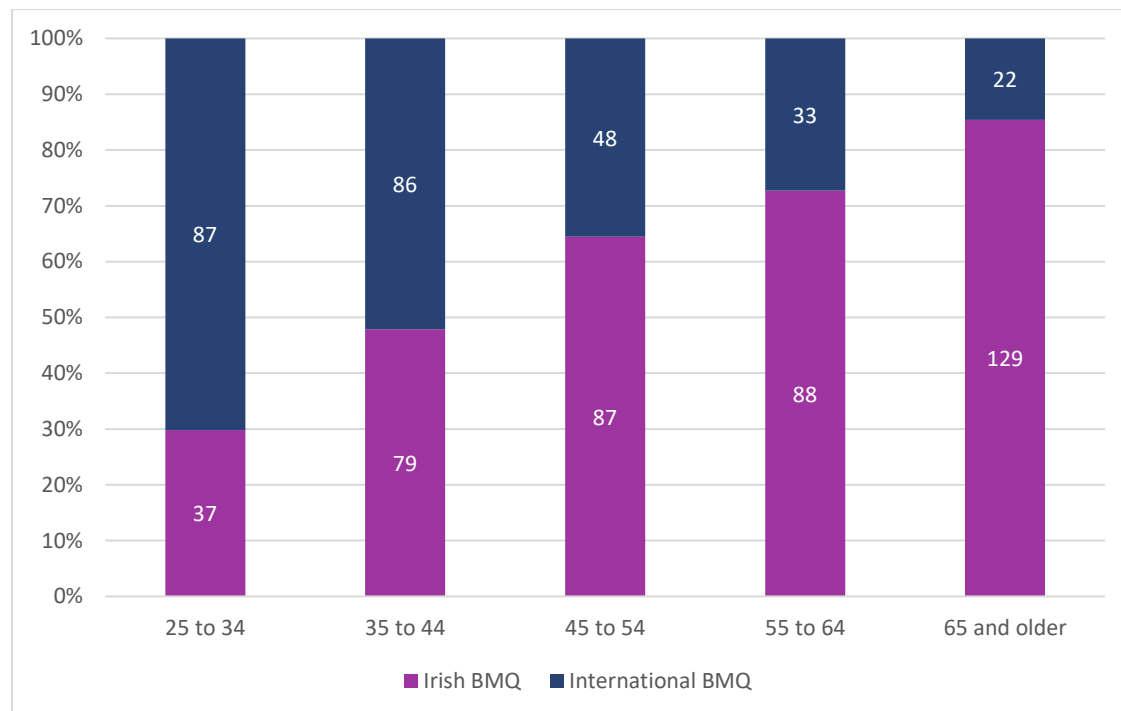
Among doctors 64 years and younger a higher proportion of doctors retaining and not clinically active were female, whereas among doctors aged 65 and older a higher proportion of doctors were male.

Figure 22. Gender By Age Category



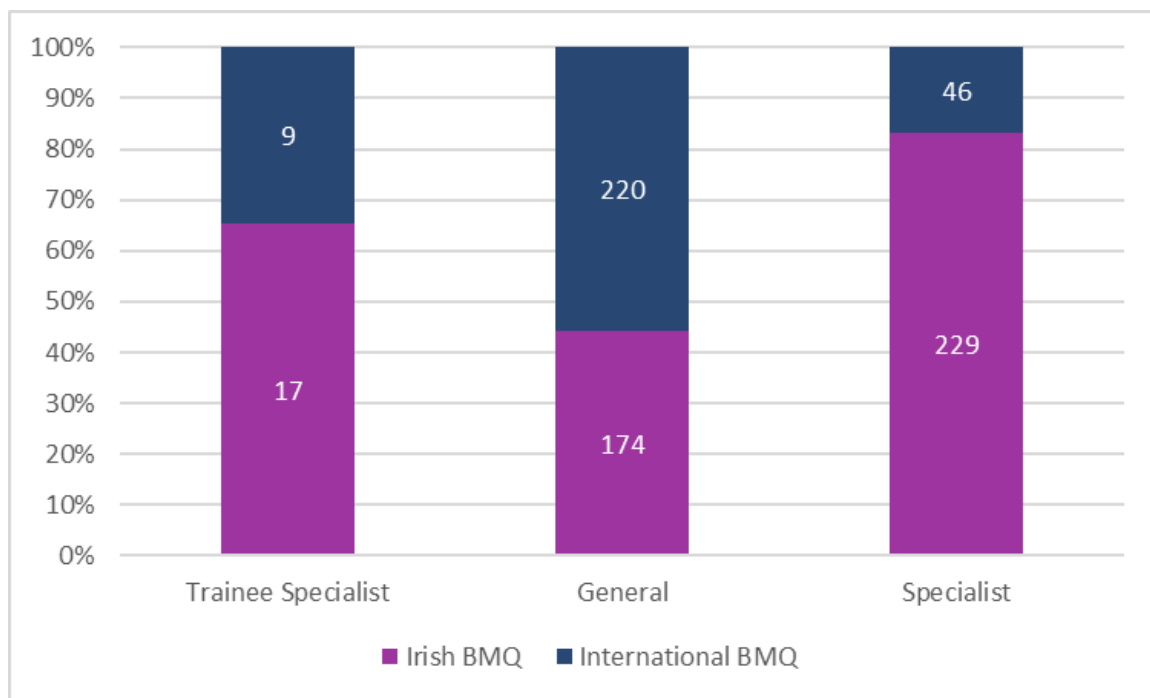
Most doctors had an Irish BMQ (N = 420, 60.3%) while 39.7% (N = 276) had a BMQ from outside of Ireland. There was a higher proportion of those aged 44 years and younger with an International BMQ and a higher proportion of those aged 55 and older with an Irish BMQ.

Figure 23. BMQ By Age Category



There was a higher proportion of specialists and trainee specialists with an Irish BMQ while there was a higher proportion of those on the general division with an international BMQ. This mirrors the distribution of doctors who retain and are clinically active.

Figure 24. BMQ By Division

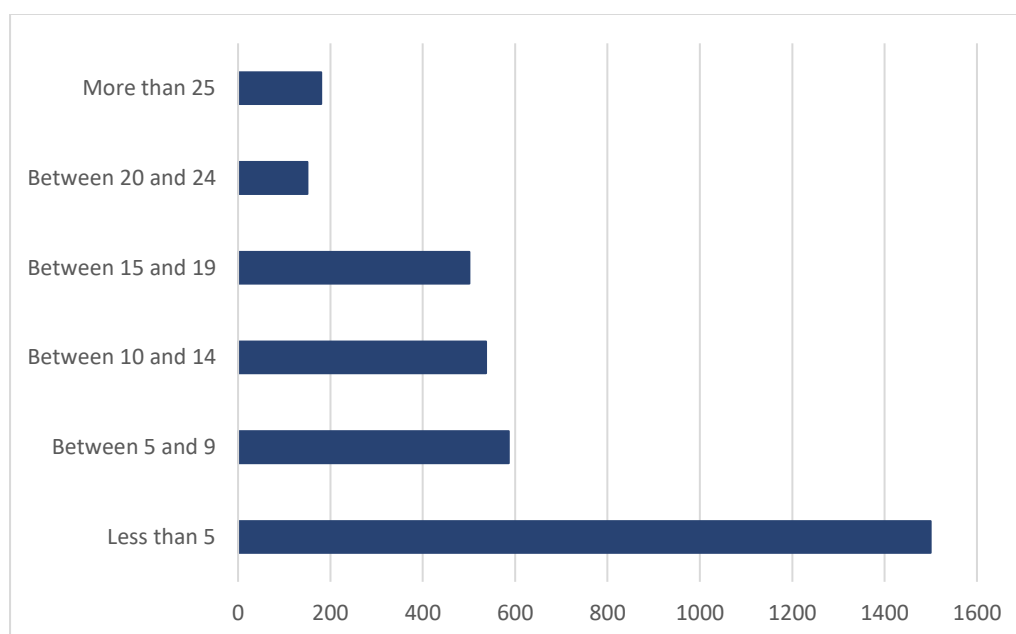


Doctors who Retain but Only Practise Abroad

In 2022, 22,317 (96.6%) of registered doctors reported being clinically active. Of those reporting that they were clinically active 3,454 (15.5%) practised outside of Ireland only.

In comparison to those who retained and were not clinically active and those clinically active in Ireland there was a much higher proportion of males (N = 2,392, 69.3%) than females (N= 1,062, 30.7%) clinically active abroad only. The majority of these doctors were registered on the general division (N = 2,335, 67.6%), followed by those on the specialist (N = 1,064, 30.8%), the trainee specialist (N = 48, 1.4%) and the supervised division (N = 7, 0.2%). Nearly half of these doctors had been registered with the Medical Council for less than 5 years (N = 1,500, 43.3%).

Figure 25. Years Practising Of Doctors Who Retain But Practise Only Abroad



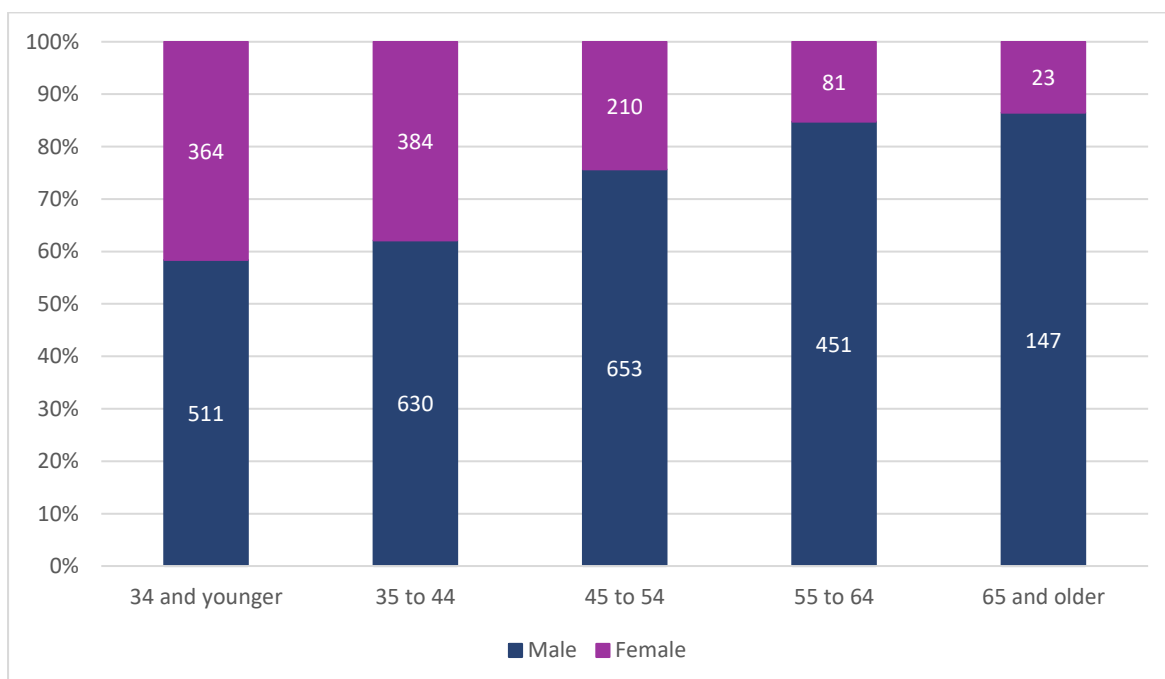
These doctors were aged between 24 and 84 (M = 44.0, SD = 11.8) years, with the majority of respondents aged between 35 and 44 years (N = 1,014, 29.4%).

Table 7. Age Categories Of Doctors Who Retain But Practise Only Abroad

Age Category	N	%
34 and younger	875	25.3%
35 to 44	1,014	29.4%
45 to 54	863	25.0%
55 to 64	532	15.4%
65 and older	170	4.9%
Total	3,454	100.0%

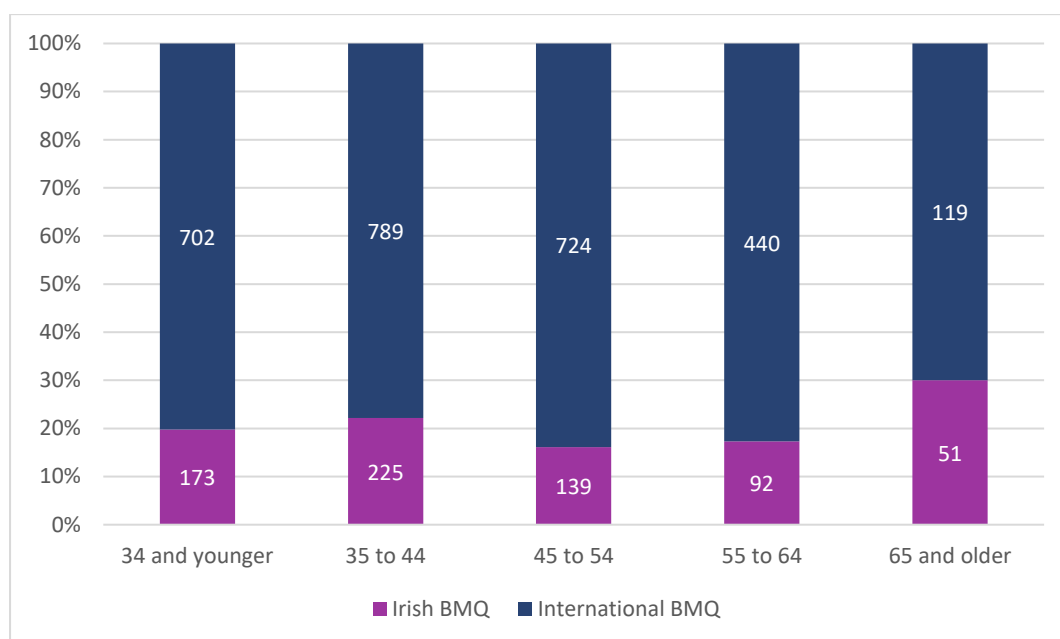
The proportion of males to females was higher in all age groups but this was more pronounced among older doctors.

Figure 26. Gender Of Doctors Who Retain But Practise Only Abroad, By Age Category



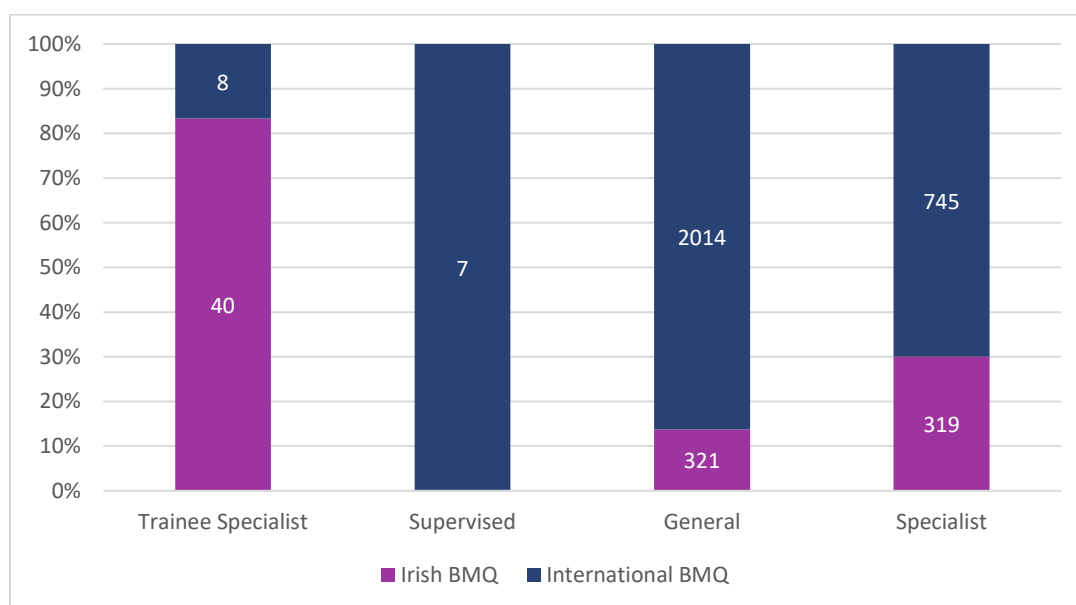
Just over 8 out of 10 doctors had an international BMQ (N = 2,774, 80.3%) while 19.7% (N = 680) had an Irish BMQ. The trend was similar across all age categories.

Figure 27. BMQ Of Doctors Who Retain But Practise Only Abroad, By Age Category



When looking at BMQ by division of doctors working abroad the trainee specialist division was the only division with a majority of doctors with an Irish BMQ

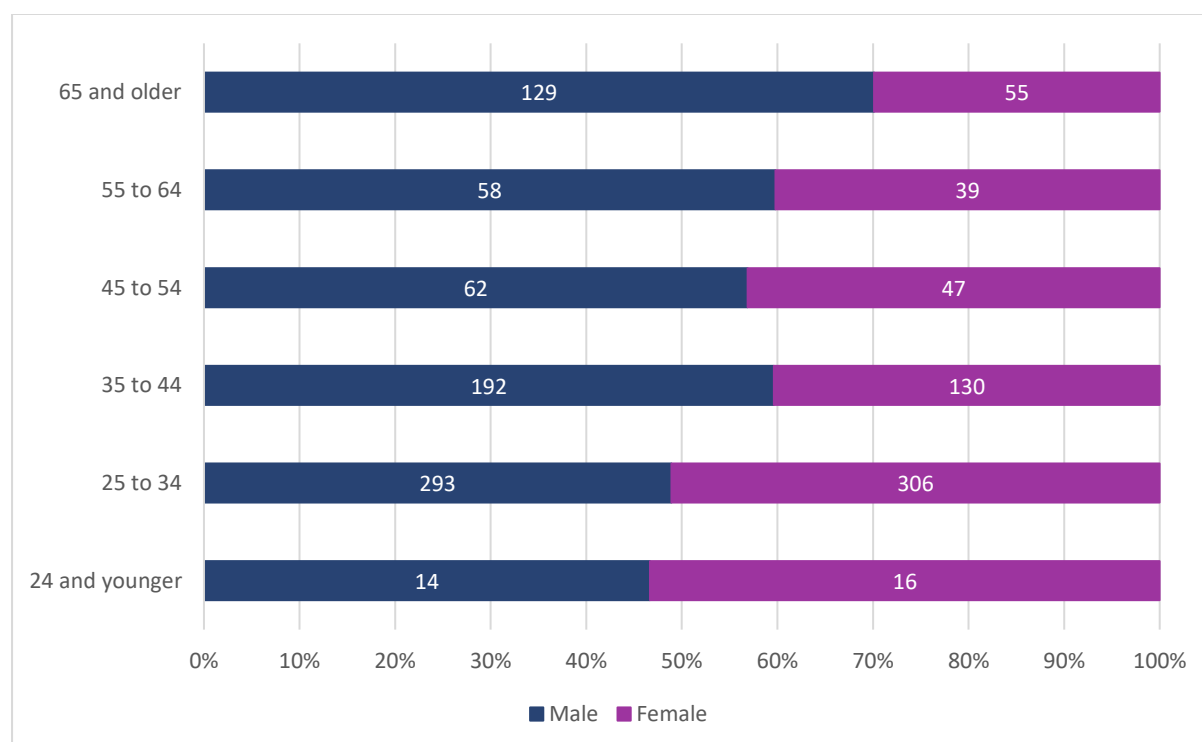
Figure 28. BMQ Of Doctors Who Retain But Practise Only Abroad, By Division



Voluntary Withdrawals

In 2022, 1,341 doctors voluntarily withdrew their registration from the Medical Council. The mean age of this cohort was 40.7 years (SD = 15.0). There were 748 males (55.8%) and 593 (44.2%) females. Figure 29 below shows breakdown of gender by age categories.

Figure 29. Gender Breakdown By Age Category



Of those who withdrew, 586 (43.7%) obtained their BMQ in Ireland, whereas 755 (56.3%) were graduates from medical schools abroad (more details of the specific BMQ categories shown in Figure 30). In terms of the division from which they withdrew their registration, 838 (62.5%) were registered on the General Division, and 361 (26.9%) on the Specialist Division (Figure 31).

Figure 30. BMQ Category Of Doctors Withdrawing

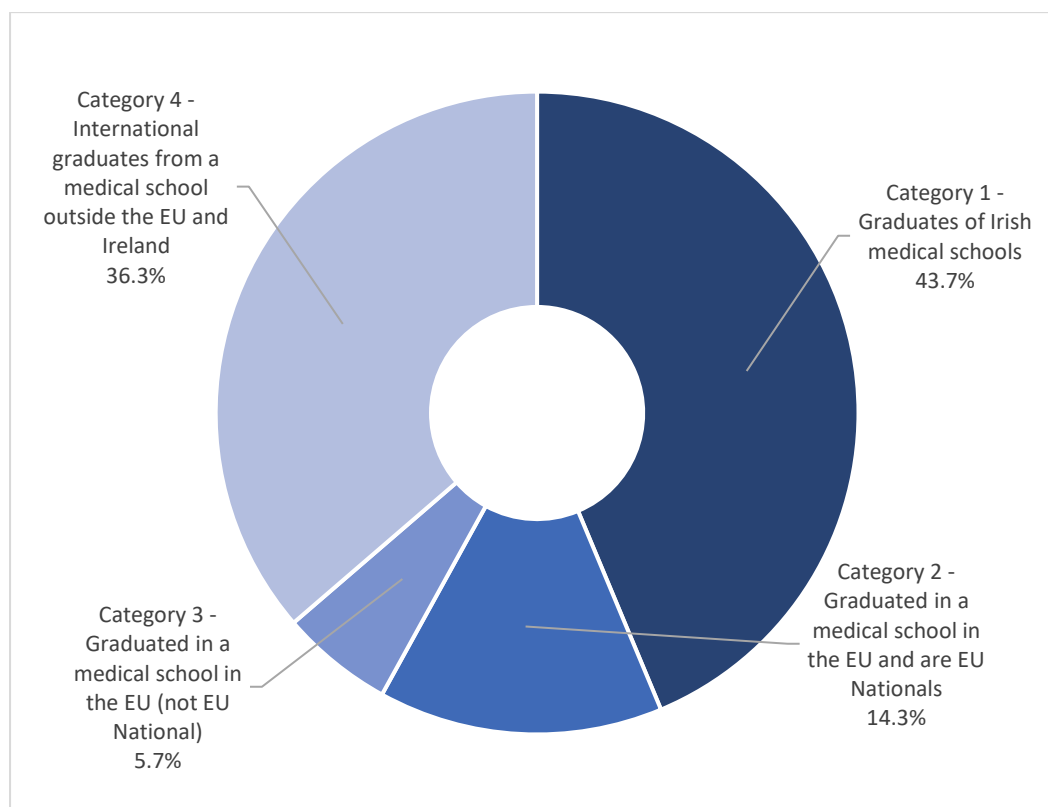
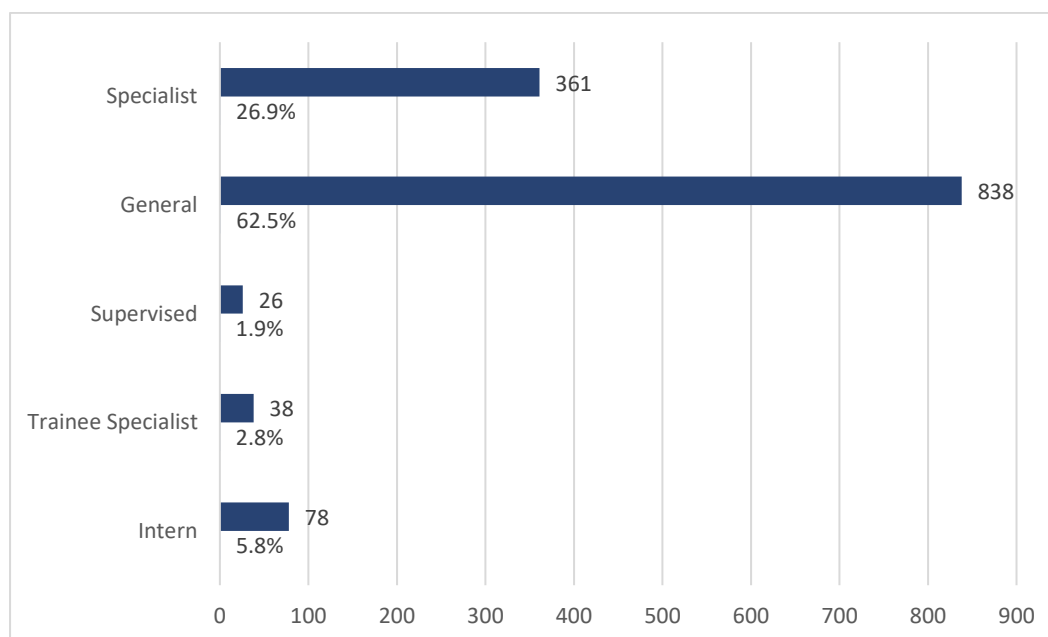


Figure 31. Division Of Register From Which Doctors Withdrew



Reasons for withdrawal

When asked about the reason for their withdrawal request, 844 doctors (62.9%) answered they wished to practise medicine in another country, 227 (16.9%) wished to stop practising medicine and 270 (20.1%) reported having some other reason for voluntarily withdrawing. Doctors were then asked to provide more details on their withdrawal reason. Table 8 shows further details on more specific reasons doctors were able to select from while completing the withdrawal survey.

Table 8. Withdrawal Reason Details

Withdrawal Reason	N	%
I have some other reason for making a voluntary withdrawal	351	26.2%
I have family/personal reasons for making a voluntary withdrawal	300	22.4%
I am retiring	194	14.5%
There are limited career progression opportunities available	163	12.2%
I am changing to a role that doesn't require me to be registered	87	6.5%
N/A or Missing	60	4.5%
I feel I can earn more abroad	56	4.2%
The working hours expected of me here are too long	47	3.5%
My workplace is understaffed	23	1.7%
I do not have flexible training options	21	1.6%
The quality of training available to me here is poor	14	1.0%
I am expected to carry out too many non-core tasks	11	0.8%
My employer does not support me in my work	10	0.7%
I am not respected by senior colleagues	4	0.3%
Total	1,341	100.0%

Among those doctors who completed the voluntary withdrawal survey, 616 (45.9%) offered detailed explanations on the reasons for withdrawing their registration. These were compiled for thematic analysis to obtain more in-depth qualitative feedback of these doctors' experiences. Several themes emerged from the analysis: practising abroad, limited career progression, poor working conditions, registration requirements, personal or family reasons, and other. In many cases, these themes were interrelated and doctors pointed out multi-faceted reasons for withdrawing. For example, many doctors were practising abroad because of poor working conditions in Ireland, limited training opportunities, or family reasons. Each theme is described below individually:

Practising abroad

Many doctors commented that they withdrew as they have been practising medicine outside of Ireland for some time, or that they were planning to relocate to another country and practise there, either temporarily or on a long-term basis. By far the most popular destinations were the United Kingdom and Australia, followed by Canada and New Zealand. Some doctors working in Northern Ireland might have been paying fees to both the Irish Medical Council and the General Medical Council and decided to practise solely in the UK. Other common destinations were international medical graduates' native countries, such as Pakistan, where many doctors returned to work or to obtain further specialised training after their Irish experience.

"I have moved to Australia to gain more experience, but I do plan on returning to the Irish healthcare system in the next few years"

"I have been practising in the UK since July 2021 and had previously retained registration with the Medical Council in the event that I practiced in Ireland but will most probably not do so until after 2024"

"I am practising medicine in Pakistan and currently have no plans to start practising medicine in Ireland in near future."

Limited career progression

Frequently, doctors simply stated that they were not able to find or secure a job, despite trying for several months and in numerous hospitals. This was particularly the case for those who wanted to further their careers in certain specialties. Furthermore, this situation was even more difficult for IMGs, many of whom mentioned that they had even fewer opportunities for career progression and were at a disadvantage in terms of obtaining training posts. These reasons drove many doctors to look for opportunities in the UK and other destinations.

"I would like to voluntarily withdraw from the register because, mysteriously and unfairly, I've not found any vacancy so far in Ireland, despite being a highly qualified doctor and keeping tirelessly applying for a long time!"

"Ireland offers no career progression for foreign doctors. No matter how hard we work. I am very disappointed with my experience there. A change is required to train in order to retain much needed doctors."

“I have found very limited job opportunities, with HSE and in private sector. I will keep an eye on the market, and hopefully return should things change. For now, I am continuing my practice in the UK.”

Poor working conditions

Poor working conditions were frequently cited and discussed in detail as reasons for leaving Ireland and seeking opportunities abroad. Doctors mentioned working long hours in understaffed, stressful, and unsupported clinical environments, being badly paid, and having poor training opportunities. Many others stated that the healthcare system was not flexible, that hospital rotations were inconvenient, hospital systems (IT, digital records) were lacking and sub-standard, and that work-life balance was poor. Furthermore, some doctors even mentioned that they were bullied and were the subject to discrimination, exploitation, and racism.

“I feel Ireland does not offer attractive training opportunities or working conditions. We are overworked, overtaxed and underpaid, with very little work-life balance. Irish doctors will continue to leave the country if this does not change.”

“Chronically under-staffed and overworked, with excessive working hours and no support from senior management. Also the lack of and the flexible training and the requirement to move city every year is too punishing when I want to buy a house and start a family”

“The policies of Ireland towards non eu doctors are very discriminatory. There’s no equality in term of opportunities for non eu doctors. Nepotism and racism prevail in this system which makes it very suffocating for doctors like myself. I feel I was just being used by this system and in return I was not given any career progression opportunities. Lack of supervision and lack of support are just tip of the iceberg.”

Registration requirements

Unlike the previous themes relating to the healthcare system and the clinical working environment, this theme captures different aspects pertaining to the registration process and requirements that deter some doctors from maintaining their registration. In particular, financial reasons were cited frequently, with doctors pointing out that fees were financially prohibitive, especially for those who were also practising in the UK or other jurisdictions and therefore had to pay additional fees (e.g. GMC). Some doctors wished that there was a special registration type with a “dormant” option that would allow a reduced fee to maintain registration while not practising in Ireland. Finally, requirements for professional competence, CPD, and audits, as well as insurance and indemnity issues, were often

cited as superfluous depending on the personal working situation of the doctor, prompting some to withdraw from the Register.

“Presently working in another country. Plus the yearly fee is very high. You should apply the same process as GMC UK where they have minimum fee of 180 pounds a year to maintain your name on register and pay full fee only when one wants to work in uk and want to have license to practice in uk!!!”

“The expense of remaining on the Register and the challenges of meeting CPD requirements etc mean that I have little alternative to voluntarily removing myself from the Register.”

“It is too expensive to pay registration in 2 different countries. Another reason is that in order to maintain registration in IMC I need also registration with Professional Competence Scheme, which is valid only when working in Ireland and since March 2022 I have been working overseas”

Personal or family reasons

Personal or family reasons were also very commonly cited as reasons for withdrawing from the Register. Some mentioned having to care for family members, maternity leave, and childcare. Others stated that they were experiencing health issues preventing them from practising, that they were having a break from their careers, or in the case of older doctors, that they were retiring. Yet other doctors were withdrawing because they were accompanying their partners, sometimes also doctors, and moving abroad.

“I can't work at the moment as I am currently needed at home on a full-time basis to look after two very frail and elderly relatives.”

“I have a one year old who is still very attached to me and I would like to spend some more time with him before I restart practising again. I have another kid 6 years old who also needs some more of my time this year.”

“I live in Switzerland now and I wish to continue here since my wife works here at present.”

Other

Finally, there were miscellaneous reasons for withdrawing not necessarily captured by the previous themes, such as seeking post-specialist training abroad (completing fellowships abroad, or engaging in

any other higher training specialisation). It was very common for doctors to express their intention to come back to the Register in the future.

“I am currently completing a clinical fellowship in Canada. I will not be practising medicine in Ireland whilst I am on Fellowship here.”

“Moving to UK for fellowship in paediatric anesthesia, fully intend to return to Ireland for consultancy”

Additional reasons included no longer practising medicine, international doctors finishing a fellowship in Ireland, having returned to the Register only temporarily as part of the Covid-19 response, and registration being no longer required given the working arrangements or situation of the doctor. The temporary COVID Register ran until June 2022 and some doctors voluntarily withdrew prior to this.

“I retired in June 2018, I reregistered on a temporary basis in 2021 in order to be involved in the vaccination process during the Covid pandemic. I am now not practising medicine.”

“I am starting a business that does not require me to be registered with the Medical Council”

Key Insights for Workforce Planning

Analysis of data identifies several salient issues that can potentially impact on the protection of the public and patient safety. Lack of appropriate training and career development opportunities, excessive working hours in contravention of the EWTd, capacity and doctor shortages, attrition and withdrawal from the register are all key challenges.

1. The General Division

Doctors on the general division account for a substantial proportion of all clinically active doctors in Ireland. The general division was created as a transition for doctors progressing from internship to the trainee specialist division, and does not offer specialist training. In 2022 31.2% (N = 5,870) of doctors who were clinically active in Ireland were on the general division. 4% of doctors self-reporting their role as hospital consultants were on the general division. A cohort of these doctors correctly meet required standards to be registered on the specialist division, however, they are incorrectly registered as they did not 'grandfather' or transfer to the specialist division as they were entitled.

2. Non-Consultant Hospital Doctors Not-in-Training

Immense over-reliance on NCHDs to deliver care has implications for public protection safety. Reliance on NCHDs continues to be a feature of the Irish healthcare system. Ireland has an excess of NCHDs not in training, relative to specialists. In 2022, there were a similar number of NCHDs in training as not in training. Current training, supervision and working conditions of NCHDs pose serious implications for patient and professional safety.

The NDTP reports that in comparison with international healthcare systems, while Ireland has the lowest ratio of consultants per 100,000 of the population, it has the highest ratio of NCHDs. Alongside Australia, Ireland compares poorly with almost 50% less consultants in the workforce per head of population and approximately 30% more NCHDs.⁶

The majority of NCHDs not in training are doctors who have trained overseas, a pattern that has remained unchanged in recent years. Many are employed in in Model 2 and Model 3 hospitals (NDTP,

⁶ *Medical Workforce Report 2020-2021*. (2021). HSE - National Doctors Training & Planning Retrieved from <https://www.hse.ie/eng/staff/leadership-education-development/met/ed/rep/medical-workforce-report-2020-21.pdf>

2022)).⁷ Training requirements of NCHDs on the general division need to be urgently addressed by investing in professional development opportunities. The NDTP's Strategic Plan 2022-2027 outlines the need for an increased number of NCHDs in recognised postgraduate training programmes and a reduction of the number of NCHDs on the general division.⁸

3. International Medical Graduates

The majority of NCHDs not in training are doctors who have trained overseas, and many have not been able to access specialist training in Ireland. Ireland is highly dependent on international medical graduates and has the fourth highest proportion of foreign educated doctors within OECD member states.⁹ Among OECD countries Ireland has an average number of doctors per capita, however, in 2016, the proportion of foreign-trained doctors rose to 42% which was the 2nd highest in the OECD.¹⁰ In 2021, according to the OECD this remained around 40%, and Ireland was ranked the fourth highest country.¹¹ This is despite Ireland being in the top three OECD countries for training medical graduates per capita.¹²

Research indicates that foreign-trained doctors report entering posts in Irish hospitals with the expectation of career progression or accessing specialist training. Lack of progression opportunities and fear of deskilling prompt many to actively seek to migrate or return to their home country within a few years of arriving in Ireland.¹³

The WHO Code of Conduct stresses equal employment opportunities for international graduates. This must be addressed by increasing the specialist training places for international doctors, leading also to a reduction in the prevalence of NCHDs and an increase in specialist numbers. In the 2021/22 academic year there were 1,403 medical student places available in the Irish Higher Education System. However

⁷ *Medical Workforce Report 2022-2023*. (2023). HSE - National Doctors Training & Planning Retrieved from <https://www.hse.ie/eng/staff/leadership-education-development/met/plan/ndtp-medical-workforce-report-2023.pdf>

⁸ *Strategic Plan 2022–2027 Delivering The Medical Workforce Vision For Ireland*. (2022). National Doctors Training & Planning

⁹ Hynes, T., & O'Connor, P. (2023). *Spending Review 2022: An Analysis of Medical Workforce Supply*.: Research Services & Policy Unit, Department of Health, Irish Government Economic and Evaluation Service

¹⁰ Heffron, M., & Socha-Dietrich, K. (2019). The Irish paradox: Doctor shortages despite high numbers of domestic and foreign medical graduates. *Recent Trends in International Migration of Doctors, Nurses And Medical Students*. OECD.

¹¹ OECD. (2023). *Health at a Glance 2023: OECD Indicators*. O. Publishing. <https://doi.org/10.1787/7a7afb35-en>

¹² Ibid.

¹³ Heffron, M., & Socha-Dietrich, K. (2019). The Irish paradox: Doctor shortages despite high numbers of domestic and foreign medical graduates. *Recent Trends in International Migration of Doctors, Nurses And Medical Students*. OECD.

due to the large proportion of non-EU students (46%) and availability specialist training capacity many of these graduates do not progress to become consultants in Ireland.¹⁴

4. Workforce Shortage

Hospital Consultants

Consultant posts are not being adequately filled and this impacts on supervision and training for trainees and junior doctors. From the Medical Council perspective, if doctors do not receive appropriate education and training, patient and professional safety issues arise. In order to have suitably trained applicants for new and replacement consultant posts, a sufficient number of consultants need to be trained and retained. In July 2022 the government announced an agreement with Irish medical schools to increase the number of places available for EU students by 200 over the next 5 years.¹⁵ While this is a welcome expansion in the number of training places available, further training and progression supports for doctors are essential.

General Practice

Ireland has an ageing GP workforce, and a substantial proportion of GPs are close to retirement age. Of concern, the NDTP indicates that the number of training posts available to train for general practice was substantially less than the number of GPs required to facilitate the roll-out of Sláintecare.⁷

Increasing pressure on GP is a result of a growing and ageing population, increased patient multi-morbidity, ageing GP workforce with significant numbers facing retirement, poor morale and burnout. Another factor is a policy shift towards primary care with more complex conditions and chronic illnesses being managed in the community. This is evident as Sláintecare reforms are reorientating the health service towards primary care, which demands an increased and expanded GP workforce.

Current and future shortages of GPs are a concern, especially in rural and remote areas. Ireland has a high rural based population and currently 15% of the workforce are in a rural GP practice.¹⁶ A shortage of GPs means that waiting times for GP appointments are becoming an issue, with at least 10% of the

¹⁴ Hynes, T., & O'Connor, P. (2023). *Spending Review 2022: An Analysis of Medical Workforce Supply*.: Research Services & Policy Unit, Department of Health, Irish Government Economic and Evaluation Service

¹⁵ Extra 200 medicine places in Irish medical schools over the next 5 years announced by Ministers Harris and Donnelly. (2022). In: Department of Health.

¹⁶ *Shaping The Future: A Discussion Paper On The Workforce And Workload Crisis In General Practice in Ireland*. (2022). Irish College of General Practitioners

population being unable to sign up with a GP because their patient lists are at capacity, according to the IMO.¹⁷

Recently, the non-EU rural GP programme, one of several solutions proposed by the ICGP to the current GP workforce and workload crisis, has sought to attract non-EU doctors to rural general practice and has garnered nearly 50 participants during its initial phase, while a further 20 doctors have successfully concluded a three-day induction program. This two-year HSE-funded programme combines frontline GP work with mentoring and training at designated practices. While this initiative seeks to help ease the workforce crisis in some parts of rural Ireland, longer term solutions to the shortage of GPs are crucial.

5. Non-compliance with EWTD

In 2022, over one-quarter of all doctors reported working more than 48 hours on average a week, in contravention of the EWTD. Of concern, over half of all trainee specialists report working more than 48 hours per week. Excessive working hours was especially evident among those working in neurosurgery, and cardiothoracic surgery.

Your Training Counts 2019-2020 points to a correlation between hours worked per week and presentation of perceived stress, burnout, compassion satisfaction, secondary traumatic stress and perceived organisational support. Excessive work hours are demonstrably associated with attrition, stress, burnout and are predictive of adverse event involvement (Humphries, 2021), which can undermine patient safety.

6. Doctor Attrition

Our research indicates that family and personal issues, lack of training opportunities, inadequate resourcing and work conditions are all reasons for doctor withdrawal. In 2022, there were 1,341 voluntary withdrawals recorded, an increase of 359 on 2021's figure

The retention of fully trained specialists is a key concern, and in 2022 27% of doctors withdrawing from the Register were on the specialist division, an increase from 25% in 2021. This cohort of doctors represent a significant investment by the state. While there are benefits to having some doctors

¹⁷ IMO AGM to focus attention on Capacity Crisis across the Health Services. (2023). <https://www.imo.ie/news-media/news-press-releases/2023/imo-agm-to-focus-attention/>

receive further training and experience abroad, there are roles with lower levels of specialisation, for which foreign training may not be as necessary.¹⁸

Compared to other European countries, Ireland has a relatively high emigration rate.¹⁹ Recent research²⁰ indicates that 45% of junior doctors planned to remain in Ireland, while 35% planned to leave but return later. Of concern, 17% planned to leave and not return and 3% planned to stop practising medicine altogether.

The UK, Australia and Canada emerges as the main beneficiary of Ireland's shortcomings as these are the destinations of preference for most of those withdrawing from the Irish Register. Doctors report that they can access training programmes abroad, and this has been facilitated by the experience they gained practising in Ireland. Improvements in working conditions, perceptions of training and support of well-being are essential to retaining doctors.²¹ Ensuring the system is safe and attractive to stay and work in is crucial to producing and maintaining a sustainable workforce into the future.

¹⁸ Pierse, T., Morris, R., OToole, L., Kinirons, B., & Staddon, E. (2023). The retention of training doctors in the Irish health system. *Irish Journal of Medical Science (1971-)*, 1-8.

¹⁹ Socha-Dietrich, K., & Dumont, J.-C. (2021). International migration and movement of doctors to and within OECD countries-2000 to 2018: Developments in countries of destination and impact on countries of origin.

²⁰ Brugha, R., Clarke, N., Hendrick, L., & Sweeney, J. (2021). Doctor retention: a cross-sectional study of how Ireland has been losing the battle. *International journal of health policy and management*, 10(6), 299.

²¹ Cronin, F., Clarke, N., Hendrick, L., Conroy, R., & Brugha, R. (2019). The impacts of training pathways and experiences during intern year on doctor emigration from Ireland. *Human resources for health*, 17, 1-10.

Concluding Comments

The salient concerns, outlined above, present potential risks for the health and well-being of doctors and their capacity to provide safe, quality patient care. Both patients and doctors face risks in a healthcare system that is under-resourced, and this can lead to avoidable fatalities and poor health outcomes. Recent research indicates that delays to timely admission from emergency departments are known to harm patients.²² There is 1 estimated death for every 82 patients whose time to inpatient bed from time of arrival at an emergency department (ED) is delayed beyond 6-8 hours and for every 72 patients waiting 8-12 hours. More than 1,200 people may have died unnecessarily in Irish hospitals in 2022 winter period up to the end of January because of delayed admission and treatment linked to overcrowding, according to the HSE data.

As the regulatory body for doctors, the Medical Council has a statutory role in protecting the public and ensuring patient safety by promoting the highest professional standards amongst doctors practising in Ireland. The Council is legally obliged to ensure that issues compromising patient safety are mitigated and addressed. The Council recognises that it has a responsibility to protect patient safety and support the medical profession in a transparent, respectful and fair manner. Through the application of right touch regulation, the Council can and will influence required changes within the health system to support provision of safe quality health care.

The Medical Council believes that Ireland must develop a vision for the healthcare workforce, and this should include carefully considering how doctors can be best employed to provide quality and sustainable patient care. The potential risks to patient safety and doctor well-being indicated by the findings presented in this and prior iterations of the Workforce Intelligence Report necessitate a comprehensive workforce framework. The adverse impact of the COVID-19 pandemic on the Irish healthcare system, in conjunction with pre-existing workforce issues, have compounded the need for the development of an integrated healthcare workforce strategy.

The key issues identified from the data analyses in this report can support effective planning and development of a strong and sustainable medical workforce that can provide safe, high quality sustainable patient care. Gathering and using robust, quality data to develop actionable insights to improve training and patient safety is crucial, and this should be a collaborative and shared effort across all stakeholders. Indeed, the Council is leading the umbrella of Health and Social Care Regulatory Forum to address workforce issues that present significant risk to the provision of safe, quality patient

²² BMJ. (2022). *5 hour + emergency care wait before admission linked to heightened death risk*. BMJ. Retrieved 30 November 2023 from <https://www.bmj.com/company/newsroom/5-hour-emergency-care-wait-before-admission-linked-to-heightened-death-risk/>

care. This Forum is seeking to develop integrated data linkages across regulatory bodies and development of a common minimum workforce dataset.

Workforce planning should be understood as a continual and dynamic process, that adapts to a changing environment. Significant drivers of change to the workforce will also need to be addressed, and this includes population ageing and epidemiology, current levels of service utilisation, current unmet demand, and technology and e-health. With the co-operation of all our stakeholders, the Council is willing to play an engaging and supportive role in the development and implementation of a strategic workforce framework for doctors, as an integral part of a wider healthcare workforce strategy.

Appendices

Appendix A

Table A1. Gender Of Doctors Retaining Their Place On The Register

	N	%
Male	12,747	55.2
Female	10,361	44.8
Total	23,108	100.0

Table A2. BMQ Of Doctors Retaining Their Place On The Register

	N	%
Irish BMQ	13,037	56.4
International BMQ	10,071	43.6
Total	23,108	100.0

Table A3. Division Of Doctors Retaining Their Place On The Register

	N	%
Trainee Specialist	3,170	13.7
Supervised	162	0.7
General	8,637	37.4
Specialist	11,139	48.2
Total	23,108	100.0

Appendix B

Table B1. Country Of Qualification For International BMQ Holders Who Were Clinically Active In Ireland

Country	N	%	Country	N	%	Country	N	%	Country	N	%
Pakistan	1,583	22.7%	Ukraine	32	0.5%	Sri Lanka	7	0.1%	Lebanon	~	~
Sudan	887	12.7%	Bangladesh	31	0.4%	Malta	7	0.1%	Norway	~	~
United Kingdom	721	10.3%	Saudi Arabia	28	0.4%	Ethiopia	6	0.1%	Yugoslavia	~	~
Romania	481	6.9%	Cuba	27	0.4%	Serbia	6	0.1%	Unknown	~	~
South Africa	426	6.1%	Venezuela	25	0.4%	Turkey	6	0.1%	Algeria	~	~
India	315	4.5%	Dominican Republic	23	0.3%	Haiti	5	0.1%	Antigua and Barbuda	~	~
Egypt	259	3.7%	France	21	0.3%	Kazakhstan	5	0.1%	Armenia	~	~
Hungary	202	2.9%	Malaysia	18	0.3%	Zimbabwe	5	0.1%	Congo	~	~
Poland	201	2.9%	Portugal	17	0.2%	Trinidad and Tobago	5	0.1%	Costa Rica	~	~
Nigeria	131	1.9%	Colombia	16	0.2%	Austria	~	~	Denmark	~	~
Czech Republic	121	1.7%	Peru	15	0.2%	Myanmar	~	~	Dominica	~	~
Bulgaria	99	1.4%	Jordan	14	0.2%	Grenada	~	~	Finland	~	~
Spain	98	1.4%	Bahrain	12	0.2%	Kenya	~	~	Guatemala	~	~
Iraq	95	1.4%	Brazil	12	0.2%	Bolivia	~	~	Honduras	~	~
Libyan Arab Jamahiriya	91	1.3%	Mexico	12	0.2%	Cayman Islands	~	~	Japan	~	~
Ireland	83	1.2%	Oman	12	0.2%	Ecuador	~	~	Malawi	~	~
Italy	79	1.1%	Philippines	12	0.2%	Nepal	~	~	Curacao	~	~
Slovakia	78	1.1%	United States of America	11	0.2%	Saint Kitts and Nevis	~	~	Panama	~	~
Germany	70	1.0%	Argentina	10	0.1%	Slovenia	~	~	Saint Lucia	~	~
Lithuania	56	0.8%	Belgium	10	0.1%	Switzerland	~	~	Saint Vincent and the Grenadines	~	~
Croatia	54	0.8%	Republic of Moldova	10	0.1%	Tanzania	~	~	Uganda	~	~
Latvia	52	0.7%	United Arab Emirates	10	0.1%	Albania	~	~	The Republic of Macedonia	~	~
Russian Federation	43	0.6%	Iran, Islamic Republic of	9	0.1%	Chile	~	~	Uruguay	~	~
China	42	0.6%	Belarus	8	0.1%	El Salvador	~	~	Uzbekistan	~	~
Netherlands	38	0.5%	Kyrgyzstan	8	0.1%	Georgia	~	~	Zambia	~	~
Greece	35	0.5%	New Zealand	8	0.1%	Ghana	~	~	Netherlands Antilles	~	~
Australia	33	0.5%	Yemen	8	0.1%	Indonesia	~	~	Total	6,968	100.0%
Syrian Arab Republic	33	0.5%	Canada	7	0.1%	Kuwait	~	~			

~ Countries with less than 5 doctors have been suppressed in the above table.

Appendix C

Table C1. Further Details On Current Area Of Practice Among Doctors Clinically Active In Ireland

Area of practice	N	%
Anaesthesiology	1,234	6.6%
Emergency Medicine	939	5.0%
General Practice	4,953	26.4%
Cardiology	312	1.7%
Clinical Genetics	14	0.1%
Clinical Neurophysiology	15	0.1%
Clinical Pharmacology and Therapeutics	19	0.1%
Dermatology	213	1.1%
Endocrinology and Diabetes Mellitus	196	1.0%
Gastroenterology	264	1.4%
General (Internal) Medicine	1,453	7.7%
Genito-Urinary Medicine	20	0.1%
Geriatric Medicine	394	2.1%
Infectious Diseases	101	0.5%
Intensive Care Medicine	90	0.5%
Medical Oncology	194	1.0%
Military Medicine	13	0.1%
Nephrology	128	0.7%
Neurology	141	0.8%
Palliative Medicine	138	0.7%
Pharmaceutical Medicine	34	0.2%
Rehabilitation Medicine	44	0.2%
Respiratory Medicine	259	1.4%
Rheumatology	135	0.7%
Tropical Medicine	6	0.0%
Obstetrics and Gynaecology	735	3.9%
Occupational Medicine	156	0.8%
Ophthalmology	171	0.9%
Paediatrics	1,091	5.8%
Pathology	622	3.3%
Psychiatry	1,490	7.9%
Public Health Medicine	295	1.6%
Radiology	625	3.3%
Sports and Exercise Medicine	28	0.1%
Cardiothoracic Surgery	100	0.5%
General Surgery	891	4.7%
Neurosurgery	60	0.3%
Ophthalmic Surgery	122	0.6%
Oral and Maxillo-Facial Surgery	29	0.2%
Otolaryngology	177	0.9%
Paediatric Surgery	32	0.2%
Plastic, Reconstructive and Aesthetic Surgery	162	0.9%
Trauma and Orthopaedic Surgery	513	2.7%
Urology	167	0.9%
Neurophysiology	~	~
Total	18,779	100%

* 60 doctors did not provide information on their current area of practice and are excluded from this table.

~ Values less than 5 doctors have been suppressed in the above table.

Table C2. Breakdown Within Psychiatry, Pathology, And Paediatrics

Specialty	Sub-Specialty	N	%
Psychiatry		1,490	7.9
	Child and Adolescent Psychiatry	215	1.1
	Psychiatry	1,122	6.0
	Psychiatry of Learning Disability	46	0.2
	Psychiatry of Old Age	107	0.6
Paediatrics		1,091	5.8
	Neonatology	115	0.6
	Paediatric Cardiology	18	0.1
	Paediatrics	958	5.1
Pathology		622	3.3
	Chemical Pathology	19	0.1
	Haematology (Clinical and Laboratory)	200	1.1
	Histopathology	236	1.3
	Immunology (Clinical and Laboratory)	24	0.1
	Microbiology	133	0.7
	Neuropathology	10	0.1
Radiology		625	3.3
	Radiation Oncology	93	0.5
	Radiology	532	2.8

Appendix D

Table D1. Self-Reported Role By Age Category In Clinically Active Doctors

Role	34 and younger		35 to 44		45 to 54		55 and older		Total	
	N	% within role	N	% within role	N	% within role	N	% within role	N	% within role
Non-consultant hospital doctor, in training	2,806	70.90%	1,026	26.00%	108	2.70%	16	0.40%	3,956	100.00%
Non-consultant hospital doctor, not in training	1,767	44.60%	1,409	35.60%	526	13.30%	257	6.50%	3,959	100.00%
Hospital Consultant	74	1.50%	1,261	26.40%	1,835	38.40%	1,605	33.60%	4,775	100.00%
Other Consultant or Specialist	10	1.60%	150	23.70%	182	28.70%	292	46.10%	634	100.00%
General practitioner	379	8.40%	1,459	32.20%	1,195	26.30%	1,503	33.10%	4,536	100.00%
Community Health Doctor	10	4.80%	65	31.30%	55	26.40%	78	37.50%	208	100.00%
Public Health Doctor	20	11.80%	41	24.10%	44	25.90%	65	38.20%	170	100.00%
Healthcare related management and administration	6	9.40%	10	15.60%	18	28.10%	30	46.90%	64	100.00%
Others	87	21.50%	100	24.70%	92	22.70%	126	31.10%	405	100.00%
Total	5,159	27.50%	5,521	29.50%	4,055	21.70%	3,972	21.20%	18,707	100.00%

* 132 doctors did not provide information on their self-reported role and are excluded from this table.

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